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RFI Response: Health Technology Ecosystem

Smart health. Made human.

Sharecare response to Centers for Medicare & Medicaid Services [CMS-0042-NC]

Patients and Caregivers

Patient needs

PC-1
What health management or care navigation apps would help you understand and manage your (and your loved ones) health needs, as well as actions you should take?

People and caregivers struggle daily with the fragmented healthcare system interacting with multiple patient portals, insurance websites, out-of-date provider directories, lack of transparency in healthcare costs, confusion about their health plan coverage/benefits, and stand-alone digital solutions that don't talk to each other. The proliferation of smartphone apps and on-demand services across every industry has raised the bar for what a "good" experience should look like in healthcare. Particularly after the pandemic accelerated telehealth and digital health usage, patients are now demanding personalized support available 24/7. Surveys consistently show that nearly **76% of consumers want more control over healthcare costs**, and **91% find transparency tools helpful** (htt1) in managing their care. Further, **61% of consumers consider online appointment scheduling extremely or very important**, and **77% prefer to complete pre-visit forms digitally** (htt2), which are clear indicators that people desire and will use tools that empower them to self-serve and navigate care on their own terms.

There is a clear need for an easy-to-use unifying navigation layer that can sit on top of these disparate systems and provide a one-stop shop experience to help people live a healthier, happier life.

This need is not theoretical; it's measurable. The U.S. healthcare system wastes an estimated **$760 to $935 billion annually** (htt3), with a significant portion attributed to administrative complexity, inefficient care coordination, and preventable service duplication. Over **89% of Americans** (htt4) search for health information online before consulting a provider, reflecting a massive need for trusted, accessible tools to answer health questions and guide next steps. Even something as basic as finding an in-network provider remains a challenge because as much as **30% of provider directory data is inaccurate or outdated** (htt5), leading to delays in care and poor consumer experiences. Moreover, patients often have no way to verify whether a specific provider, at a specific location, is covered under their plan, or what that the visit or procedure will cost. These ambiguities can prevent care entirely.

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Essential capabilities for a one-stop digital health experience
To address these challenges and meet consumer expectations, an ideal health navigation platform must deliver on interconnected capabilities including:

- **End-to-end navigation clarity:** An effective health platform must provide more than generic information. It must integrate plan-specific data and guide users through the entire care journey. This includes surfacing next steps that are aligned to the user's benefits: confirming whether a service is covered, identifying in-network providers, showing real-time availability, verifying referral or authorization requirements, and estimating out-of-pocket costs. All of this must be presented seamlessly, in one flow, so users aren't forced to navigate multiple disconnected systems. The capability to combine clinical guidance with insurance verification and care logistics in real time transforms a passive tool into an actionable, benefit-aware navigation experience. This clarity reduces delays, minimizes administrative burden, and empowers users to act with confidence.
- **Conversational navigation and AI-driven search:** People should be able to ask questions or describe symptoms in everyday language. Natural language AI and proven clinical models should work together to provide personalized, understandable answers, guiding users to the right level of care or the next step without overwhelming them with medical jargon or outdated, incorrect information
- **Coverage-aware guidance and real-time cost clarity:** Patients need to know whether a test, prescription, procedure or provider is covered, and what it will cost before they act. A platform should verify insurance eligibility in real time, surface formulary alternatives, and offer accurate cost estimates to help avoid surprise bills and reduce financial anxiety.
- **Smart eligibility verification and benefit navigation:** To eliminate guesswork and reduce administrative friction, a platform should enable dynamic, real-time verification of plan benefits and eligibility down to the procedure and provider level. This includes surfacing copays, deductibles, and authorization status based on a member's plan, location, provider network, and service type. This also empowers caregivers. For example, a family member assisting a Medicare, Medicaid, or Exchange beneficiary can view a unified dashboard with verified benefits and access rules across multiple services, enabling timely coordination and reducing delays in accessing appropriate care.
- **Federated identity & smart eligibility infrastructure – Flexible, future-ready access:** AskMD supports a federated identity model that unifies insurance verification, medical records, claims data, biometrics, and social determinants of health (SDOH) into a single, secure user profile. This identity can be authenticated through multiple pathways, including government-backed credentials (e.g., Login.gov), biometric options (e.g., facial recognition, CLEAR, Amazon One palm scan), or traditional MFA login, allowing for flexible but trusted access across health systems. At the core of the system is a verified insurance credential, which functions as a dynamic identity anchor, linking a person's coverage details, plan-specific rules, and eligibility history across payors and providers. This allows digital health tools like AskMD to deliver real-time benefit verification, surfacing copays, deductibles, and authorization requirements based on the provider, procedure, and location. It reduces guesswork, minimizes delays, and ensures that

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patients and caregivers are navigating care with up-to-date, plan-specific intelligence. The infrastructure is modular and CMS-aligned, allowing states and federal partners to integrate their own identity technologies while maintaining a consistent access and verification framework. Whether accessed through biometric scan or a Verified Insurance ID, the system enables secure, scalable, and policy-ready interoperability that powers seamless navigation and equitable access to care.

- **Personalized health profile with integrated data:** A comprehensive profile should combine data from insurance claims, EHRs, wearable devices, and self-reported inputs to present a real-time, longitudinal view of an individual's health. This integration enables smarter recommendations and contextual prompts based on the whole person. CMS's Blue Button API offers a foundational standard for enabling Medicare, Medicaid, and Exchange beneficiaries to access their claims data in a machine-readable format. When integrated into a unified navigation platform, this type of data improves personalization and makes it possible to surface relevant insights without requiring users to start from scratch.
- **Proactive care and wellness management:** A digital health platform shouldn't only respond to health needs; it should anticipate them. This includes generating timely, personalized nudges for preventive screenings, medication fills, and chronic condition follow-ups, tailored to a user's health status and benefit design. To maximize impact, these prompts should be paired with behavioral incentives. Studies from the CMS Innovation Center, the NIH, and large employer-sponsored health programs have consistently shown that even small financial or non-financial rewards can significantly increase adherence to recommended care, particularly for screenings, chronic disease management, and wellness activities. Embedding reward logic directly into the digital experience reinforces positive behaviors at the point of decision, helping users stay engaged while reducing preventable costs to the system. AskMD is designed to integrate with Medicare Advantage supplemental benefits, ACO REACH initiatives, and quality-linked rewards programs enabling continuous engagement tracking that can support reimbursement alignment, gap closure, and future program evaluation.
- **Seamless access to services and support:** Navigation must lead directly to action. People should be able to schedule telehealth or in-person visits, access digital therapies or coaching programs, and coordinate with caregivers through one unified interface, reducing the need for multiple apps or disjointed workflows.

Together, these functions transform a health app from a static information portal into a dynamic digital health assistant that reduces complexity, closes gaps, and drives follow-through.
We believe that Sharecare's AskMD brings these capabilities together in a consumer-ready experience that serves Medicare, Medicaid, and Exchange beneficiaries, caregivers, and other users navigating complex health needs. It is more than an AI symptom checker, it is a personalized digital front door, designed to unify fragmented systems and remove friction from every step of the healthcare journey. The collective capabilities of AskMD empowers people and their caregivers to not only to ask, "what should I do?", but to confidently answer, "where should I go, is it covered, how much will it cost, and how do I take the next step?"

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An AI front door to personalized health guidance

Sharecare's AI-driven solution turns vague questions into confident next steps. Underpinning AskMD's performance is a clinical large language model (LLM) trained specifically on real-world patient journeys and medical decision-making, not just open web data. In benchmark testing, this model consistently outperforms general-purpose LLMs in both accuracy and safety, generating recommendations that align with clinical best practices and avoiding hallucinations or inappropriate advice. In plain terms, it's like giving the AI a medical degree, allowing it to understand context, urgency, and nuance the way a clinician would. A grandmother worried about her persistent cough can describe her symptoms in plain English and immediately receive a personalize triage recommendation backed by clinically validated logic. The platform uses an explainable AI engine that transparently shares the rationale behind each recommendation. It also accounts for urgency and triggers appropriate escalation to in-person or virtual care when needed.

Unlike current symptom checkers, AskMD retains user context and builds longitudinally. It adapts responses based on prior interactions, known conditions, and life events, creating a smarter and more personalized experience over time. Its design supports a wide range of literacy levels and language preferences, using simplified terms, visual decision trees, and accessibility-first UX to ensure comprehension.

AskMD's AI models are also aware of care gaps and will flag when follow-ups are overdue, or new concerns may signal worsening health. The result is guidance experience that is not only immediate and accurate but also deeply tailored, anticipatory, and compassionate.

A passport for interoperability

Sharecare's AskMD removes one of the most frustrating pain points for users: understanding what's covered, what isn't, and why. The platform performs real-time insurance eligibility and benefit checks for Medicare, Medicaid, and commercial plans. It parses payor responses down to the procedure, provider, and place-of-service level. It provides copays, deductibles, authorization requirements, and referral needs in clear, member-specific language.

It also eliminates the need for paper-based intake by enabling users to initiate a digital experience with minimal information, such as name, date of birth, and zip code. If the member eligibility data is incomplete or rejected, the system uses intelligent fallback logic, retry protocols, and alternative identifiers to match members to the correct plan. For in-person care, the platform generates QR-enabled digital insurance cards that providers can scan to auto-populate eligibility and benefit information into their systems. This streamlined approach replaces the outdated clipboard experience in the provider's office, where patients are asked to recall medication lists, allergies, and prior visits, which often results in errors, omissions, or delays. By digitizing intake and aligning clinical, benefits, and identity data up front, AskMD reduces friction, improves data accuracy, and ensures users receive personalized next steps from the start of their care journey.

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When denials or barriers to access occur, such as prior auth requirements or out-of-network warning, AskMD captures the exact coverage rule, attaches it to the user's record, and prepares context for potential appeals. This real-time detection and documentation can reduce downstream disputes and supports care continuity.

By integrating payor-specific rules directly into the care guidance process, the AI driven solution eliminates guesswork and reduces preventable administration delays. Users and caregivers can act with clarity and confidence, knowing which options are approved and affordable.

A dynamic personal health record with agentic AI orchestration

Sharecare's AskMD turns data into action. The platform aggregates FHIR-based EHR data, C-CDA documents, insurance claims, lab results, self-reported inputs, and wearable device data into a single, longitudinal profile. This dynamic record is not static, rather it's continuously enriched and monitored by orchestrated AI agents.

For example, if a lab result suggests a worsening condition, the person can be prompted to schedule a covered visit, adjust goals, or flag the need for intervention. Agents also detect changes in behavior, like decreased activity or medication nonadherence, and adjust coaching accordingly. Every recommendation is informed by both clinical guidelines and the user's preferences, goals, and social context.

Caregivers are provided oversight through multi-profile dashboards. Authorized family members can view benefits, track appointments, and manage health responsibilities across dependents, aging parents, or other individuals. Role-based access ensures security while enabling coordination and peace of mind.

From navigation to action

Navigation alone is not enough. Execution is critical. Sharecare's AskMD helps users not only find what they need, but take meaningful, well-informed action, whether that means booking an appointment, enrolling in a benefit, or getting support for a social need. It includes an integrated provider search agent that dynamically scores options based on network participation, quality ratings, availability, location, and benefit alignment. Users see ranked options like: "Dr. Patel, primary care, in-network, $0 copay, available tomorrow at 10am."

From there, people are enabled with one-tap scheduling for telehealth or in-person care, along with enrollment in coaching programs, benefit changes, or digital therapies. Every service displays cost, coverage, and access logistics up front, removing ambiguity.

Providers are prepared in advance. When a person schedules care, AskMD sends pre-visit intake forms, eligibility confirmation, and patient-reported data directly to the clinician's system, reducing administrative friction and improving visit efficiency.

For users with unmet social needs, the solution surfaces wraparound services such as transportation, food access, and caregiving support based on eligibility, geography, and health plan resources. This integration of medical and non-medical care reinforces CMS's goals for whole-person health.

To increase engagement and close care gaps, outreach is personalized around plan-specific rewards and incentives, surfacing opportunities when users are most likely to act. If a member is booking a visit or exploring benefits, Sharecare's AskMD can flag supplemental benefit offerings like dental cleanings, fitness memberships, or meal support that they haven't used yet. This aligns with CMS's new requirement for a mid-year enrollee notification of unused supplemental benefits, helping ensure beneficiaries are aware of what is available and can use it proactively at the point of care.

The Sharecare solution also contributes to CMS's goals around eliminating fraud, waste, and abuse by proactively reducing preventable costs and ensuring that care is both necessary and appropriately delivered. By verifying eligibility and benefit details in real time (before a visit is scheduled), AskMD helps prevent inappropriate referrals, unnecessary out-of-network charges, and duplicate services. Its centralized health profile and agentic coordination layer ensure that key documentation (like prior labs, diagnoses, and authorizations) is shared across encounters, reducing redundant diagnostics and improper billing. When care is denied, it captures the policy rationale and prepares documentation for appeal, supporting both compliance and member rights. These safeguards align with CMS's push for administrative efficiency, proper payment, and better care outcomes at lower cost.

CMS's upcoming National Provider Directory creates a mission critical foundation and AskMD's functionality transforms that infrastructure into a real-time personalized pathways to care. Instead of simply listing providers, Sharecare has the ability to overlay eligibility data, benefit design, and quality indicators to surface the most relevant, in-network, and cost-appropriate options for each user.

Built for trust, built for scale

- **Security and compliance:** Sharecare's AskMD is built with leading healthcare standards at its core (HIPAA, FHIR, HL7, OAuth2, and FedRAMP-aligned protocols) ensuring sensitive information stays secure. Every interaction is logged, traceable, and protected, meeting the rigorous expectations of CMS-regulated programs. Whether a user is checking benefits, accessing health records, or scheduling care, they can trust their data is safeguarded every step of the way.
- **Access controls and user authorization:** Only the patient or an authorized caregiver has access to personal health information. The platform uses OAuth2 identity verification and role-based access rules to ensure that no third party (especially one without a direct care relationship) can see or misuse a user's data. This means users can safely manage their health or a loved one's health, without fear that their information is being sold, scraped, or used in ways they never agreed to.

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- **Omnichannel access:** AskMD meets users where they are, whether that's on a smartphone at home, through a caregiver on a desktop, or in a provider's office. This flexibility ensures continuity of experience and empowers users of all digital literacy levels to engage in their care.
- **CMS-ready infrastructure:** Sharecare's AskMD isn't retrofitted for the public sector, it's built for it. The platform supports Medicare transitions, Medicaid redeterminations, and integrates with federal and state systems for eligibility, claims, and care coordination. Its infrastructure is designed to scale with CMS's evolving mandates and priorities, ensuring that innovation doesn't come at the expense of compliance or reliability. It supports interoperability mandates, redetermination workflows, coverage transitions, and integrates with state and federal eligibility and claims infrastructure.

The healthcare system doesn't need more apps. It needs clarity, simplicity, and action. Sharecare's AskMD delivers an integrated experience that makes health and coverage information usable, timely, and personal. It replaces confusion with confidence, removes barriers to care, and guides individuals through every step with precision. Whether someone is managing chronic conditions, helping a loved one, or simply trying to understand what's covered, AskMD turns questions into answers and intent into action. This is the digital front door CMS has been envisioning: one that empowers people, reduces waste, and restores trust. It changes the healthcare experience all together for the better.

PC-1

a) What are the top things you would like to be able to do for your or your loved ones' health that can be enabled by digital health products?

Patients and caregivers most want to conveniently coordinate care and make informed decisions. This includes understanding health conditions, knowing what next steps to take (e.g. see a specialist, get a lab test, try self-care), and having help carrying out those steps. Key capabilities would be:

- Symptom triage and decision support, so the user can input symptoms or questions and get personalized guidance instead of Googling in confusion.
- Insurance navigation & eligibility verification, ensuring real-time clarity on coverage and costs for any recommended action (avoiding phone calls to insurers and surprise bills).
- Care coordination tools, including scheduling appointments, finding the right doctor or service, setting up medication reminders or therapy follow-ups.
- Centralized health information to provide easy access to one's records, lab results, medication list, etc., in one app, to inform any recommendations.
- Ongoing wellness support that not only tracks progress on health goals and delivers reminders for preventive care or screenings but also adapts based on risk signals and clinical context. Digital tools should be able to detect patterns that may indicate worsening conditions, prompt evidence-based interventions, and ensure that wellness activities are clinically relevant, not just engagement for engagement's sake. These tools

must operate across preferred channels (text, app, or voice) to maintain accessibility and adherence.
- Behavioral incentives or rewards to encourage follow-through, such as completing preventive screenings, adhering to medication, or closing care gaps, which can reinforce positive health behaviors and reduce downstream costs.
- Connected programs and services that allow individuals to access multi-modal care that addresses specific health and social risks for members, including integrated self-guided digital therapies and multi-modal lifestyle and condition coaching, case and care management services, and caregiver support programs.

All these tasks can be enabled by modern digital products. AskMD was explicitly designed to cover these needs: it combines an AI-driven symptom checker with real-time benefit checks and appointment scheduling, unifying what otherwise might require several separate apps or phone calls.

PC-1

b) If you had a personal assistant to support your health needs, what are the top things you would ask them to help with? In your response, please consider tasks that could be supported or facilitated by software solutions in the future.

If people had a dedicated personal assistant for their health, their requests wouldn't be complicated; they'd be practical, personal, and urgent. They'd ask for help making sense of symptoms, finding the right doctor, managing appointments and medications, understanding what's covered by insurance, and making sure nothing falls through the cracks.

But today, patients and caregivers are often forced to take on that job themselves. The system assures they'll act as their own coordinator, researcher, and advocate, without the tools or support to do it well. Digital health assistants can fill the gaps, simplify what's overwhelming, and help people stay on track.

Below are examples of what people would want their personal assistant to do, and how technology can help:

- **"Tell me what to do when something's wrong"**
  Whether it's chest pain, dizziness, or a confusing lab result, people want answers. A digital health assistant can triage symptoms using clinical decision support, explain possible causes in plain language, and recommend appropriate next steps, whether that involves seeing a doctor, waiting it out, or calling 911.
- **"Find me the right doctor and help me book it"**
  This includes identifying in-network providers, verifying that a service is covered, checking availability, and even offering telehealth if it's faster and appropriate. Instead of being sent to a list of links, the assistant should connect the dots and close the loop.

- **"Remind me about what matters and make it easier to follow through"**
  People forget about appointments, refills, screening, and exercise. Smart reminders that adjust based on condition, age, and behavior can help users stay on top of what matters most, from managing diabetes to scheduling a flu shot.
- **"Tell me what this means and what it costs"**
  Whether it's a lab result, a diagnosis, or an insurance EOB, people want clarity. Digital assistants can interpret test results, translate clinical language into plain terms, and estimate out-of-pocket costs using real-time benefit verification.
- **"Keep all my health information in one place"**
  From medications to medical history to past procedures, people don't want to log into five portals or carry paper records. A centralized digital health record, pulling from both clinical systems and user input, helps ensure continuity and reduces duplicative care.
- **"Support me when I need help at home"**
  Sometimes we need hands on care. An assistant should recognize when someone needs help with daily activities and connect them to trusted services. When a user mentions recovering from surgery or needing help getting to appointments, the system can offer caregiver support, matching them with a credentialed home aide, enabling two-way scheduling, and providing oversight tools for families and care managers.
- **"Help me care for someone else"**
  For adult children caring for aging parents or spouses helping each other through illness, caregiving is a complex, emotional job. A digital assistant should allow proxy access, shared dashboards, real-time notifications, and guided support so caregivers can act with confidence and coordination.
- **"Help me learn while I manage my health"**
  People often want to understand not just what to do, but why it matters. A digital assistant should offer context-sensitive educational content, explaining chronic conditions, treatment options, or the importance of follow-up care. For example, when someone reaches the age where a colonoscopy is recommended, the digital assistant should notify the person, explain the purpose of a colonoscopy, what to expect, and how it helps detect preventable disease. This kind of embedded education builds trust and empowers better decisions.

These are real, everyday needs. And they're increasingly solvable with technology. Sharecare's AskMD, combined with their CareLinx business (a national network of credentialed in-home caregivers), already supports many of these tasks today. It helps people understand symptoms, navigate their benefits, and schedule appropriate care. CareLinx brings in-person support directly into the home when more than digital help is needed. Together, they form a hybrid model that blends smart software with human care, aligned to what people actually need.

Digital health assistants even provide predictive and proactive care with the ability to flag risks based on subtle patterns, offer next steps before issues escalate, and serve as a constant partner across the care journey.

For Medicare, Medicaid, and Exchange beneficiaries and caregivers, the value is less confusion, better coordination, and more confidence navigating a system that rarely feels designed for them.

For the overall healthcare system, the value is a system that prevents more, reacts less, and spends smarter. It's a shift from episodic, high-cost care to continuous, value-aligned engagement, benefiting not only individuals, but providers, payors, and public programs like Medicare and Medicaid.

PC-2

Do you have easy access to your own and all your loved ones' health information in one location (for example, in a single patient portal or another software system)?

a) If so, what are some examples of benefits it has provided?
b) If not, in what contexts or for what workflows would it be most valuable to use one portal or system to access all such health information?
c) Were there particular data types, such as x-rays or specific test results, that were unavailable? What are the obstacles to accessing your own or your loved ones' complete health information electronically and using it for managing health conditions or finding the best care (for example, limitations in functionality, user friendliness, or access to basic technology infrastructure)?

Many patients and caregivers currently struggle with fragmented health information, scattered across multiple patient portals and paper records. Ideally, one should have easy access to all personal health data in a single, unified location. For example, an app or platform that aggregates data from different providers, insurers, and the patient themselves.
Sharecare's AskMD is built to integrate with both clinical data (from providers/HIEs) and claims/insurance data (from Medicare or other payors) using modern interoperability standards. It supports all CMS-required data fields for patient access, including claims details, encounter information, medication lists, and even provider directory info by consuming the FHIR APIs that health plans are now required to offer under CMS regulations. In other words, it can act as an authorized third-party application to retrieve a beneficiary's Medicare/Medicaid/Exchange claims history, current medications, coverage information, etc., and present it in one place. This empowers patients to make better decisions, since they can finally "see the whole picture" of their healthcare in one view.

Beyond clinical records, patients and caregivers also need real-time and accurate access to insurance information to support everyday health decisions. AskMD surfaces eligibility status, benefit coverage details, prior auth requirements, and real-time out-of-pocket cost estimates at the point of need. This includes plan-specific nuances such as whether a provider is in-network, if referrals are required, and coverage for particular services, all based on the individual's

insurance ID, location, and service context. These insights reduce confusion, prevent claim denials, and help patients make financially informed decisions before accessing care.

If a patient or caregiver today has easy access to all health info in one location (for example, via a consolidated personal health record app), the benefits are significant. They can quickly reference medical history, share accurate information with new providers, and track care for chronic conditions without flipping between systems. One concrete benefit is avoiding duplicate tests or procedures. When all past results (like x-rays, lab results) are readily available, a new doctor can avoid ordering something unnecessarily. It also saves time and reduces frustration. Caregivers managing an elderly parent's care, for example, have a single login to see medications, recent visits, and upcoming appointments for all providers involved, rather than juggling multiple portals for each specialist. AskMD demonstrates such benefits. By aggregating a user's data (with consent) from insurance claims to recent lab results into its guidance engine, it not only shows the data to the patient but actively uses it to tailor recommendations. For instance, if all health information is in one place, the app can notice a pattern (such as multiple ER visits for asthma in the past year) and prompt a discussion about better asthma management or schedule a specialist, something that might be missed if data is siloed.

Unfortunately, many people still do not have all their health information easily accessible in one spot. In those cases, certain workflows would greatly benefit from a unified system. For example, in care coordination contexts, if an elderly patient is discharged from a hospital and a caregiver is trying to organize follow-up care, having the hospital discharge summary, medication changes, and upcoming appointments all in one app (rather than some on paper and some on a portal) would be invaluable.

Another scenario is chronic disease management: patients with diabetes often see multiple providers (primary care, endocrinologist, ophthalmologist). A single personal health record app could let them, and their caregivers see all relevant data (latest lab results, medication adjustments, eye exam findings) in one timeline, making it easier to manage the illness.

Medication management is another workflow, using one system to see medications prescribed by different doctors prevents dangerous duplications or interactions.

A unified portal or app would also simplify emergency situations, as caregivers could quickly pull up an accurate medication list and problem list for EMTs or ER doctors, which is especially critical for Medicare, Medicaid, or Exchange beneficiaries who might be on many medications. There are notable gaps in certain data types that patients often encounter even when using current portals. For instance, imaging (x-rays, MRIs) and their reports are frequently not accessible or require separate logins to radiology systems. Test results from labs outside the primary health system might be unavailable. Even within integrated portals, historical data from prior providers might not be loaded. Patients often face obstacles like needing to request CDs of images or paper records for old procedures. Another missing piece can be insurance-related data, such as explanations of benefits or status of deductibles, which typically reside on insurer

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websites, not in provider portals. Patients are often unaware of what services are covered, the need for prior auths, or how close they are to meeting a deductible. The solution fills these gaps by serving as a dynamic bridge between the payor's backend and the patient experience.

Obstacles to accessing a complete health record electronically include technical and usability issues. On the technical side, not all providers are connected via interoperability networks yet. Smaller clinics or older records may not be digitized or shared. While large hospital systems have patient portals, those portals generally show only that system's data, and different portals don't talk to each other. A Medicare, Medicaid, or Exchange beneficiary might have one portal for their primary care group, another for a specialist group, and none of it includes their claims data or prescriptions from an outside pharmacy. From the user perspective, managing multiple logins and interfaces is burdensome, especially for those with limited tech experience. There may also be limitations in functionality or infrastructure: some patients may lack broadband or modern devices, making it hard to use advanced apps; others find portals not user-friendly (hard to navigate, medical jargon, etc.).

Another hurdle can be data fragmentation and privacy concerns. Health systems may be wary of pooling data into third-party apps, and patients might not know how to authorize it. The good news is that policy initiatives are addressing these obstacles. The CMS Patient Access API (Blue Button 2.0) already allows Medicare beneficiaries to share claims data with apps of their choice. Moreover, the emerging TEFCA aims to connect different health information networks nationwide. As AskMD's team has noted, TEFCA will enable "frictionless nationwide health information exchange" by establishing a universal floor of interoperability. This means that in the near future, a patient-facing app could query across all providers' networks (with the patient's consent) to retrieve records, instead of the patient manually collecting them. Once such infrastructure is in place, the vision of a single place for all your health data becomes much more attainable.

Having one location (app or portal) for all personal health information yields clear benefits: more efficient care, empowered self-management, and less administrative hassle. Solutions like AskMD are already moving in this direction by leveraging APIs to pull in claims and clinical data. The remaining challenges, such as connecting to every provider (which TEFCA can help with), ensuring all data types (like images) are included, and making the system extremely user-friendly are being actively worked on. Continued policy support for interoperability and digital literacy will further reduce these obstacles, so that patients and caregivers can truly access complete health information electronically and use it to improve care outcomes.

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PC-3
Are you aware of health management, care navigation, or personal health record apps that would be useful to Medicare beneficiaries and their caregivers?

CMS aims to expand access to effective digital health tools that serve the complex and diverse needs of Medicare, Medicaid, and Exchange beneficiaries and their caregivers. Most current digital health solutions are either narrow in function, lack integration with benefits and provider data, or fail to support real-world navigation needs. Few tools are optimized for the cognitive and accessibility challenges faced by older adults or designed with caregiver workflows in mind. This gap creates friction in care coordination, limits engagement, and can exacerbate health disparities.

To meet CMS's goals, tools should:

- Be interoperable, accessible, and compliant with TEFCA and Blue Button 2.0 evolution
- Offer robust decision support rooted in clinical and benefit data
- Enable seamless transitions from virtual to in-person or in-home care
- Support delegated caregiver access and multi-user profile management
- Be usable by individuals with limited digital literacy or disabilities

Sharecare's AskMD is a purpose-built platform that meets Medicare, Medicaid, Exchange, and caregiver needs heads-on. It offers:

- **Personal health record AI orchestration:** AskMD transforms the personal health record (PHR) into a dynamic assistant. Using an ecosystem of AI agents, AskMD continuously monitors health signals, anticipates needs, and guides users to next-best actions.
- **Caregiver functionality:** Caregivers can be invited to co-manage accounts, receive alerts, coordinate care, and access summaries. This feature is especially valuable in Medicare dual-eligible populations or those with chronic illness requiring support from adult children or home health aides.
- **Integrated navigation:** AskMD connects users with in-network providers, verifies coverage, flags prior auth needs, and facilitates scheduling, all grounded in real-time insurance and benefits data. It also surfaces relevant plan details such as visit limits, tiered provider networks, formulary coverage, and deductible status. By proactively identifying potential cost barriers, alternative medications, or out-oof-network risks before a visit, AskMD helps users make informed decisions and avoid care delays. This reduces friction, enhances appropriate benefit utilization, improves plan compliance, and supports value-based care goals.
- **CareLinx in-home services:** For patients with mobility limitations or post-acute needs, AskMD (through our CareLinx solution) dispatches credentialed home caregivers. This fulfills the "last mile" of digital health navigation and supports CMS's home-and community-based services (HCBS) objectives.
- **User-centered design:** The interface uses plain language, large touchpoints, and multimodal access (web, mobile, voice) ensuring that even individuals with limited digital experience can navigate their health confidently. It also supports voice-enabled access

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via Amazon Alexa, improving accessibility for users with low digital literacy or visual impairments. Journeys are designed for all coverage types and ensure equitable access by guiding users to covered services and local resources.

The solution is built to advance CMS's digital health equity and caregiver support priorities. It closes the gap between information and action, integrating clinical, logistical, and benefit-based inputs to reduce friction in healthcare decision-making. As CMS looks to modernize the Medicare, Medicaid, and Exchange experience, AskMD's comprehensive and scalable solution simplifies healthcare for complex beneficiaries, enhances outcomes through proactive guidance, and reduces unnecessary utilization through right-time, right-place navigation.

PC-4

What features are missing from apps you use or that you are aware of today?

a) If so, what are some examples of benefits it has provided? What apps should exist but do not yet? Why do you believe they do not exist yet?
b) What set of workflows do you believe CMS is uniquely positioned to offer?

Despite growing availability of health apps, very few address the needs of Medicare, Medicaid, or Exchange beneficiaries holistically. Existing tools often lack clinical intelligence, fail to account for benefit design, and do not support caregivers. They remain siloed in function and static in utility, offering information rather than action. CMS is seeking to better understand what digital services are missing from the market and how to enable more intelligent, integrated, and trustworthy solutions.

Missing features in existing digital health apps:

- Many available tools focus on isolated functions (e.g., symptom checkers, appointment booking, medication reminders), but fail to deliver a truly integrated, end-to-end experience. Features that are notably missing or underdeveloped include:
- **Agentic health navigation:** Apps rarely orchestrate next steps across a full care journey (from triage to scheduling, referrals, eligibility, and follow-up). They lack dynamic coordination and the ability to act autonomously based on real-time data and preferences.
- **Contextual personalization:** Current apps do not adequately adapt to individual social risk factors, benefits coverage, caregiving dynamics, or local resource availability.
- **Multi-modal, multi-stakeholder engagement:** Few tools effectively engage both patients and their caregivers, clinicians, and benefits administrators within the same digital ecosystem.
- **Interoperability and data portability:** Patients still struggle to access complete records across payors, providers, labs, and pharmacies, especially in ways that are actionable.
- **Integrated identity and eligibility:** A streamlined "passport for interoperability" that connects secure digital ID, real-time eligibility checks, and benefit activation remains unavailable.

- **Marketplace connectivity:** There is no comprehensive, trusted marketplace that connects users with third-party digital therapeutics, care programs, or coaching apps, which limits choice and continuity.

Why these apps don't yet exist:

- **Fragmented ownership of data and identity:** Clinical data, administrative claims, pharmacy information, and social determinants data are siloed across EHR vendors, payors, third-party service providers, and HIEs. Each entity maintains different access rules, consent mechanisms, and incentives for sharing, preventing unified user-level data flow.
- **Lack of investment in multi-agent AI infrastructures:** While single-function AI (e.g., symptom checkers, chatbots) is common, few platforms have invested in scalable, domain-specific agents that interact via shared memory and contextual reasoning. Building a robust, agentic orchestration model requires significant investment in both AI safety and operational validation, which is especially critical for guiding medically complex populations like Medicare, Medicaid, and Exchange beneficiaries.
- **Sustainability considerations for TEFCA:** Rather than viewing frameworks like TEFCA as a threat to existing health data platforms, it is vital to ensure they are integrated in ways that sustain the broader ecosystem. Health data service providers including clinical intermediaries and enterprise platforms contribute far more than basic data transmission. They enable data normalization, identity verification, consent management, and audit trails. TEFCA should complement, not displace, these value-added services, and CMS should ensure reimbursement and operational frameworks support this alignment.
- **Insufficient regulatory incentives for comprehensive, longitudinal tools:** Current frameworks reward adherence to narrow compliance metrics but offer limited incentives for tools that combine triage, navigation, care coordination, and social support. Without clear reimbursement or contracting pathways, developers may deprioritize holistic solutions.

CMS can accelerate innovation by championing workflows that connect policy, technology, and user need. Examples include:

- **Digital care plan templates:** Link diagnosis, benefits, and next steps using dynamic templates that guide timely interventions while ensuring cost-effectiveness.
- **Caregiver delegation standards:** Formalize proxy access rights and consent protocols across systems to enable transparent caregiver involvement.
- **Certification for AI navigation agents:** Just as CMS certifies EHRs, it can develop frameworks for vetting AI tools that support safe, equitable navigation.
- **Operational blueprints for HCBS and in-home care**: Standardize intake, triage, and remote coordination of home-based services to support aging in place.
- **Federated personal health record anchors:** Promote patient-controlled (vs. patient-directed), interoperable PHRs that support a longitudinal health narrative.
- **Sustained and additional reimbursement models for digital tools & care interventions:** Reinforce and establish reimbursement certainty for digital tools and

therapies, telehealth, coaching, and caregiving programs, leveraging Medicare Advantage 2025 fee schedules that include three Healthcare Common Procedure Coding System (HCPSC) supply codes for select digital therapeutics cleared or approved by the FDA under 21 CFR 882.5801, and further expanding against digital tools and care interventions covered, as well applicable segments (e.g. Medicaid).

Sharecare's AskMD addresses these gaps by combining four foundational elements into one integrated, agentic platform:

- **A dynamic AI layer:** Allows users to ask health questions in natural language. Responses are generated using medical knowledge graphs, user health data, and benefits logic, offering personalized, actionable, and clinically grounded recommendations.
- **A passport-like architecture for interoperability:** AskMD's federated identity infrastructure unifies insurance verification, medical records, benefit logic, and social context into a single user profile. The "digital health passport" enables users to authenticate through multiple pathways, including Login.gov, biometrics, or traditional credentials. It also supports delegated access for authorized caregivers. It facilitates tokenized consent, role-based access, and real-time eligibility checks across Medicare, Medicaid, and Exchange plans. This infrastructure supports CMS's goals around modular interoperability, fraud prevention, and reduced administrative burden, while preserving platform flexibility to deliver value-added services like benefit alignment, coverage rule translation, and pre-visit readiness.
- **An agentic, dynamic PHR:** Evolves the PHR into a real-time care command center. AI agents coordinate pre-visit planning, triage, care navigation, benefit usage, and follow-up, empowering patients and caregivers across settings.
- **A curated digital health marketplace:** Connects users to third-party and in-house services (e.g., coaching, rewards, in-home care). These services are matched to users through AI based on clinical need, personal preferences, and benefit eligibility.

AskMD turns information into insight, insight into navigation, and navigation into action and solves the very fragmentation that CMS seeks to address.

PC-5

What can CMS and its partners do to encourage patient and caregiver interest in these digital health products?

a) What role, if any, should CMS have in reviewing or approving digital health products on the basis of their efficacy, quality or impact or both on health outcomes (not approving in the sense of a coverage determination)? What criteria should be used if there is a review process? What technology solutions, policy changes, or program design changes can increase patient and caregiver adoption of digital health products (for example, enhancements to data access, reimbursement adjustments, or new beneficiary communications)?

b) What changes would enable timely access to high quality CMS and provider generated data on patients?

Despite a proliferation of digital health tools, adoption by Medicare, Medicaid, and Exchange beneficiaries and their caregivers remains uneven and inconsistent. Many users remain unaware of available tools or are skeptical of their value, especially when these tools are not recommended by trusted providers or integrated into health plans and benefit structures. Caregivers, who are often left out of the digital engagement design process, face additional challenges in navigating a fragmented and unvetted ecosystem. CMS seeks to increase uptake of trusted, valuable digital tools by creating a policy and incentive environment that promotes engagement, reduces friction, and signals credibility.

Capabilities and support needed: CMS and its partners must create an ecosystem in which digital health products are not only available but also trusted, effective, integrated into care pathways, and supported by aligned incentives. Key strategies include:

- **Transparent, outcome-based certification pathway:** CMS could establish a voluntary endorsement framework focused on usability, accessibility, efficacy, and interoperability. This "CMS-Endorsed" badge would signal credibility, promote safety, and distinguish high-quality tools without stifling innovation.
- **Integration into plan materials and provider workflows:** Inclusion of certified tools in Medicare plan directories, provider portals, and CMS communication channels (e.g., Medicare.gov, Medicare & You) would drive visibility and trust.
- **Reimbursement alignment for non-clinical tools:** CMS should expand reimbursement clarity for tools that meaningfully improve outcomes, even if they don't fall under traditional clinical services. This includes coaching, navigation, digital therapies, and telehealth, where appropriate.
- **Incentives for providers and health plans to drive use: CMS** can reward ACOs and Medicare Advantage plans for incorporating certified tools into care delivery models and for demonstrating measurable digital engagement by patients and caregivers.
- **Real-time data integration and user value:** Patients and caregivers will adopt tools that offer immediate and personalized value. By enabling real-time access to eligibility, out-

of-pocket costs, preventive care schedules, and benefits usage, CMS can enhance the utility of digital health platforms.

- **Education campaigns with caregiver-inclusive messaging:** Outreach efforts should include simplified, accessible content explaining what digital health tools do, why they matter, and how caregivers can be part of the experience.

CMS' role in oversight and acceleration is to accelerate responsible adoption of digital health tools, CMS can:

- **Establish a voluntary certification program:** Focused on usability, outcome alignment, AI transparency, and caregiver access.
- **Promote tools in official channels:** Highlight high-quality digital platforms in Medicare communications and plan support resources.
- **Encourage provider endorsement:** Incentivize integration of certified tools into EHRs and care workflows to support provider co-signaling.
- **Align reimbursement policies:** Create clear and consistent coverage for digital tools that enhance self-management and reduce system friction.
- **Foster education and trust-building:** Launch awareness campaigns that explain the value and safety of vetted digital tools to older adults and their caregivers.

Digital health tools have the potential to reduce healthcare costs, improve outcomes, and close gaps in access, particularly when they reach underserved or high-need populations. By certifying trusted tools like AskMD, enabling their reimbursement, and integrating them into communications and care pathways, CMS can transform digital adoption into a catalyst for health equity and system modernization. When patients and caregivers trust the tools at their fingertips and can use them easily to act on real-time data, engagement shifts from occasional to habitual, and outcomes follow.

PC-6
What features are most important to make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services?

Many Medicare beneficiaries and their caregivers face significant hurdles in using digital health products. These include limited digital literacy, vision or hearing impairments, cognitive challenges, language barriers, financial constraints, outdated technology, and low trust in unfamiliar tools. CMS seeks to identify the design features and functionality that improve usability, accessibility, and equity, especially for seniors and underserved populations.

Design principles and capabilities needed:

- **Simple, intuitive interfaces:** Digital tools must use clean layouts with large, legible text, minimal clicks to complete tasks, and intuitive navigation. Labels should be written in

plain language, not medical or technical jargon. For example, instead of "consult provider," say "talk to your doctor."

- **Multimodal input and output (touch, voice, text):** Not all seniors can or want to type. Tools should include voice input (e.g., through Alexa or Google Assistant), screen reader compatibility, and responsive design for both smartphones and computers. For those with hearing impairments, closed captions and audio alternatives should be standard.
- **Guided onboarding and help:** Apps should include interactive tutorials and real-time support features such as chat, call lines, or tooltips. A "learn-as-you-go" model helps users build confidence. On first use, the tool should explain each section clearly (e.g., "This is where you'll find your medications," etc.).
- **Proxy access and caregiver support:** Secure, permission-based systems must allow designated caregivers to view, manage, and act on behalf of beneficiaries. Caregivers should receive alerts and communicate with care teams through the app, ensuring continuity of care.
- **Culturally responsive, multilingual interfaces:** Digital health tools should offer content in multiple languages and reflect cultural nuances related to caregiving, condition stigma, and communication preferences. Personalization should include language choice and visual symbols familiar to diverse populations.
- **Personalization and relevance:** Tools should tailor content to the user's age, conditions, plan benefits, and communication style. An app for a 70-year-old should not present pregnancy content, and a caregiver should be able to manage multiple profiles (e.g., "Dad's health dashboard").
- **Real-time eligibility verification and benefit integration:** Knowing what is covered and when is crucial. Tools should integrate real-time eligibility verification and cost estimates, so beneficiaries can act with confidence (e.g., "This screening is covered 100% under your plan").
- **Appointment scheduling and end-to-end navigation:** Products should support the full care journey from identifying a health concern and verifying coverage to booking appointments and receiving follow-up reminders. This closed-loop approach reduces drop-off and confusion.
- **Offline and low-bandwidth functionality:** Apps should work in low-connectivity areas, offer offline access to important info (like care plans or med lists), and operate on older devices. A lightweight version or even SMS-based interactions should be considered for the most vulnerable users.
- **Trust signals and data privacy:** Users must feel safe. Clear disclosures on privacy, trusted affiliations (e.g., "endorsed by CMS" or "vetted by providers"), and reminders that digital tools support but don't replace doctors help build trust. These signals are especially vital for older adults wary of scams.
- **Seamless integration with providers and payors:** Integration with plan data and clinical systems allows for real-time updates, accurate benefit use, and provider-endorsed guidance. It also reassures users that the tool complements but doesn't compete with their care team.

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AskMD exemplifies CMS's vision for accessible, usable, and inclusive digital health platforms for Medicare, Medicaid, and Exchange beneficiaries and caregivers:

- **Omnichannel access:** AskMD is accessible via voice assistants (e.g., Alexa), desktop browsers, and mobile apps, all optimized for seniors with large-text formats and compatibility with assistive technologies.
- **Conversational AI and plain language:** Users interact naturally, describing health issues in their own words and receiving plain-language, benefit-informed responses. Visual cues and suggested next steps further simplify navigation.
- **Personalized experience:** AskMD remembers prior interactions, adapts to user behavior, and highlights relevant topics (e.g., screenings, chronic disease management) based on profile data. Caregivers can manage loved ones' health within the same platform.
- **Eligibility-driven decision support:** The system verifies benefits in real-time, helping users find covered services, in-network providers, and supplemental benefits without needing to navigate their insurance plan alone.
- **Care journey coordination:** From symptom input to scheduling care, AskMD supports follow-through with reminders, referrals, and follow-up tracking, enhancing adherence and engagement. It also surfaces critical benefit nuances, such as wellness eligibility, deductible thresholds, and supplemental benefit availability, in the context of a user's profile. This helps users avoid unexpected costs or administrative delays, which are particularly harmful to seniors on fixed incomes.
- **Multilingual, culturally-aware design:** AskMD supports multiple languages and culturally attuned content, ensuring broad accessibility and respect for diverse users.
- **Caregiver tools:** Shared dashboards, alert systems, and participation in agent-guided sessions enable active caregiving without compromising privacy.
- **Agentic coordination:** AI agents specializing in clinical, benefits, behavioral, and social support domains work together behind the scenes to deliver cohesive and context-aware guidance.
- **508 compliant:** AskMD is fully compliant with Section 508 of the Rehabilitation Act, ensuring accessibility for users with visual, auditory, and cognitive impairments, and meeting federal standards for digital inclusivity.

If CMS wants digital health adoption to be equitable, engaging, and impactful, it must prioritize design that respects the lived realities of its users. Tools like AskMD that are simple yet powerful, familiar yet future-ready, and personalized yet scalable offer a compelling model. By supporting policies and programs that elevate such tools, CMS can bridge the usability gap and ensure digital health reaches those who need it most.

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PC-7

If CMS were to collect real-world data on digital health products' impact on health outcomes and related costs once they are released into the market, what would be the best means of doing so?

As digital health tools become increasingly prevalent, CMS must establish a systematic, scalable framework to evaluate their real-world impact on health outcomes, engagement, cost, and equity. Traditional evaluation methods such as retrospective claims analysis or academic studies are often slow, fragmented, or disconnected from frontline usage. CMS seeks a modernized, minimally burdensome approach to post-market surveillance that delivers actionable insight while preserving innovation.

Capabilities needed:

- **Standardized encounter-level logging & metadata collection:** Digital tools should natively capture structured, de-identified data on usage, adherence, user experience, and outcomes. This includes navigation path metadata, AI agent decisions and alternatives considered, drop-off points or escalation triggers, and time to resolution or action taken.
- **Claims data linkage with app usage indicators:** CMS should require MA plans and CMMI demonstration participants to submit usage flags indicating which members use certified digital tools. CMS can then correlate usage with downstream claims including hospitalizations, ER visits, and medication adherence to quantify real-world effects without requiring app developers to share PHI.
- **Embedded patient-reported outcome measures (PROMs) and longitudinal feedback loops:** Digital health tools should integrate real-time prompts for PROMs post-intervention. These PROMs can be standardized using templates and mapped to CMS evaluation metrics.
- **AI audit trails and explainability metadata:** Where agentic AI systems are involved, platforms must provide metadata about decisions, alternatives, and beneficiary input shaping the output. This supports both safety monitoring and regulatory oversight.
- **Health equity stratification and disparities monitoring:** CMS should require that all submitted evaluation data be stratified by race/ethnicity, disability status, dual eligibility, rurality/urbanicity, and language/digital literacy.
- **Developer participation in voluntary surveillance networks:** CMS can establish a voluntary digital health surveillance network where certified tools agree to submit aggregate reports, participate in benchmarks, and share usage-linked outcomes data.
- **Integration with CMS APIs:** CMS should enable secure data transmission via APIs aligned with CEHRT and TEFCA standards to minimize reporting burden and facilitate real-time syncing.

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AskMD is natively designed to support CMS-aligned post-market surveillance through a transparent, real-time, and standards-based infrastructure:

- **Encounter-level logging:** Every interaction is logged with metadata, including user input, AI agent pathways, benefit checks, and care recommendations, creating a digital data trail. This enables CMS to trace clinical and navigational decisions to downstream outcomes and supports attribution accuracy by tying each digital encounter to the appropriate beneficiary, provider, and risk contract.
- **Dynamic PROMs capture:** AskMD prompts users at strategic points to assess current health status, rate experience, report follow-through, or indicate if the issue was resolved or additional support is needed. These are stored and mapped longitudinally creating a continuous feedback loop that captures patient-reported outcomes and experiences, offering insight into engagement, intervention impact, and care resolution.
- **De-identified usage reporting:** AskMD can share aggregate reports under data use agreements, without exposing PHI.
- **Equity lens:** AskMD's metadata includes race/ethnicity, SDOH context, digital modality, and proxy involvement, which supports equity evaluation.
- **Developer readiness:** AskMD's architecture is built for integration with CMS Blue Button, and FHIR APIs, supporting automated, low-friction data sharing and regulatory alignment.

By embedding evaluation infrastructure into digital tools like AskMD, CMS can transition from retrospective, manual review to proactive, real-time insight. This approach minimizes reporting burden, surfaces actionable feedback, supports dynamic program improvement, and ensures digital health policy advances without compromising equity or safety.

Furthermore, it reinforces the broader modernization goals laid out in the 'New Ideas for the Trump HHS' blueprint by using digital platforms not only to improve care delivery but also to streamline government efficiency and data-driven oversight.

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Data access and integration

PC-8

In your experience, what health data is readily available and valuable to patients or their caregivers or both?

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?
b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?
c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

Medicare, Medicaid, and Exchange beneficiaries generate a vast amount of health-related data across clinical, administrative, and self-reported domains. However, access to this data remains fragmented, often inaccessible to the very individuals, caregivers, and digital tools best positioned to use it for care coordination, personalization, and prevention. CMS seeks to understand what types of data hold the most value, where friction exists, and how interoperability policies, infrastructure investments, and private-sector alignment can improve outcomes, equity, and experience.

Valuable data types that are difficult to access

CMS has already made significant strides toward democratizing claims and clinical data access via initiatives like Blue Button 2.0 and ONC's Cures Act Final Rule, which require FHIR APIs for patient-mediated access. Yet several key categories of high-value data remain inaccessible or underutilized:

- **Unstructured clinical narratives:** Encounter notes, operative reports, and discharge summaries offer critical context but are often siloed in EHRs and not shared through patient-facing APIs. This contradicts the intent of the 21st Century Cures Act and the goals laid out in the CARIN framework calling for greater transparency and longitudinal access for caregivers.
- **Real-time coverage, prior authorization, and formularies:** Digital tools and consumers alike lack streamlined access to plan-specific benefit details, a gap that undermines CMS's vision of person-centered benefit navigation. Integration of administrative and clinical workflows should occur at the point of decision-making.
- **Real-time scheduling and provider availability:** While provider directories exist, they often lack real-time appointment data. The HHS Digital Health Strategy

Blueprint identifies "closed-loop scheduling" and "availability transparency" as foundational to meaningful digital engagement.

- **SDOH and functional status data:** While recognized as essential to equitable care by both CMS and CMMI's Innovation Strategy Refresh, SDOH data is not consistently captured or interoperable. Functional assessment data (e.g., ADLs) also remains trapped in disparate systems or clinical notes.
- **Remote monitoring and patient-reported data:** Despite high value for chronic disease management, integration of self-generated data from devices or symptom tracking apps remains limited in practice.

High-value data sources beyond claims and clinical records

- **Digital interaction & symptom history (AskMD):** Real-time interactions with agentic platforms like AskMD capture emerging intent, risks, and barriers that precede utilization. These behavioral signals are especially valuable in identifying care gaps before adverse events occur.
- **Caregiver logs and proxy behavior:** As emphasized in the CARIN Alliance's priorities for caregiving access, understanding caregiver engagement (particularly for beneficiaries with cognitive decline) helps reveal care dynamics not visible in claims.
- **Enrollment, eligibility, & plan rules:** Real-time data on coverage, network participation, and supplemental benefits is essential for frictionless navigation and ties to CMS's efforts to reduce denied claims, unnecessary ER use, and low-value care.
- **Provider quality and volume data:** Hospital outcomes, procedure volumes, and safety scores are not easily consumable by apps. Unlocking this information through APIs supports evidence-based provider choice, echoing ONC's interoperability goals.
- **Social and environmental contexts:** Zip-code-level or beneficiary-level SDOH indicators (e.g., food insecurity, housing stability) are rarely available within EHRs but are critical for equitable digital health interventions.

Opportunities to improve accessibility and interoperability
Policy-supported infrastructure:

- TEFCA as backbone for national Exchange: TEFCA's framework can address fragmentation by enabling apps like AskMD to fetch clinical records from multiple sources via one integration. CMS should align incentives across MA plans and fee-for-service environments to reward TEFCA participation, reinforcing HHS's Health Data Initiative. However, TEFCA must remain a voluntary means for exchange, not the sole mandatory option. Mandating TEFCA as the exclusive data exchange conduit could introduce a major single point of failure, especially if a widespread breach could compromise the entire health care data exchange system, as witnessed in 2024, jeopardizing both patient privacy and, critically, patient safety.
- **FHIR-based ecosystems with consent standards:** CMS can support widespread adoption of HL7 FHIR APIs and standardized OAuth 2.0 consent flows, particularly for longitudinal, multi-provider data sharing. These align with ONC's Interoperability Roadmap and help fulfill the Cures Act's patient access requirements. We observe

challenges with FHIR (Fast Healthcare Interoperability Resources) implementation within TEFCA. Variability in FHIR implementation across different systems can lead to interoperability issues, undermining the objective of seamless data exchange. Furthermore, the complexity and cost of FHIR implementation could be challenging for smaller rural entities, who do not have the infrastructure to support FHIR, which potentially creating disparities in participation and disproportionately benefiting larger, more resource-rich organizations, thus undermining the goal of equitable and widespread health data exchange.

Technical & governance challenges:

- **Data quality and normalization:** Even when data is accessible, lack of standardization, missing context, and free-text variability hinder research and real-time use. CMS could support the development of shared validation and normalization tools, particularly in safety-net settings.
- **Vendor lock-in and nonstandard implementations:** Many EHR vendors delay interoperability by deploying FHIR in fragmented or incomplete ways. CMS may consider a certification program for third-party digital tools, modeled after CEHRT, to encourage consistent implementation of patient data access.
- **Patient and proxy identity matching:** Interoperability is only meaningful if the right data can be linked to the right person. CMS could pilot or endorse verified digital identity approaches (e.g., Login.gov, biometric standards), while remaining sensitive to state-by-state variation in privacy norms.

Sharecare's AskMD is purpose-built to overcome the fragmentation CMS is trying to solve:

- **Multisource, real-time data Integration:** AskMD connects structured clinical data, claims, benefit logic, appointment availability, SDOH indicators, and self-reported data into one longitudinal user profile.
- **FHIR-native architecture:** It is architected for scalable interoperability, supporting patient-mediated exchange under Cures Act, automated pulls supported by the TEFCA framework, and app authorization flows aligned with OAuth 2.0 standards.
- **Proxy and caregiver data portability:** The platform honors granular permissioning for caregivers, supporting secure proxy access in line with CARIN Alliance recommendations and HIPAA-compliant delegation protocols.
- **Behavioral insights and symptom logs:** AskMD's agentic interactions generate engagement metadata, digital triage outcomes, and behavior-linked insights that can feed real-world evidence generation, which supports CMMI's call for continuous learning systems.
- **Marketplace and quality-driven provider choice:** By integrating provider quality ratings, cost information, and real-time scheduling, the platform supports smarter decision-making enabling patient empowerment.

By investing in infrastructure, APIs, and governance models that enable full-spectrum data access from clinical notes to caregiver logs and real-time eligibility, CMS can unlock a new

generation of AI-powered tools that reduce cost, improve quality, and advance equity. Sharecare's AskMD represents a blueprint for such tools: actionable, integrated, compliant, and person-centered.

PC-9

Given that the Blue Button 2.0 API only includes basic patient demographic, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

a. What difficulties are there in accessing or utilizing these data sources today?
b. What suggestions do you have to improve the Blue Button 2.0 API experience?
c. Is there non-CMS data that should be included in the API?

The Blue Button 2.0 API currently offers only a narrow slice of Medicare claims and coverage data (Parts A, B, D), limiting its value as a digital tool for navigation, fraud detection, and patient empowerment. To improve government efficiency and reduce duplicative technology investments, CMS is exploring how to make this API more comprehensive, integrated, and user-friendly—without relying on costly or underperforming bureaucratic layers.

Valuable CMS data types to add:

- **Medicare Advantage (Part C) encounter data:** Currently excluded, yet MA covers over 50% of beneficiaries. Including these encounters would allow parity in digital services and help detect over- or under-utilization patterns more easily.
- **Supplemental and plan-specific benefits:** Knowing if a beneficiary has vision, dental, transportation, OTC, or food coverage improves navigation and reduces inappropriate utilization of non-covered services.
- **Prior authorization status & plan rules:** Real-time access to plan-level utilization management rules (step therapy, prior auth, etc.) allows solutions like Sharecare's AskMD to route beneficiaries away from denied or delayed services.
- **Provider network participation & scheduling data:** Enables beneficiaries to find in-network, accepting providers and avoid out-of-network charges.
- **Out-of-pocket status & subsidy eligibility:** Transparency around cost-sharing, LIS status, and coverage phase (e.g., donut hole) helps prevent surprise bills and non-adherence.
- **Appeals & grievance data:** Signals where friction points exist in the system, enabling proactive app-based resolution or education.
- **Eligibility & enrollment events:** Real-time verification of plan switches, dual-eligible status, and coverage effective dates helps prevent fraud and avoid duplicated enrollment.

Difficulties in accessing/utilizing these sources today:

- APIs are fragmented, delayed, or not published.
- ONC-led standards (e.g., TEFCA or EHR vendor APIs) do not cover much of this administrative data.
- Inconsistent formats across MA plans and lack of CMS-published integration guidance.
- Patients and developers face high friction navigating consent and access, particularly across agency lines.

Suggestions to improve the Blue Button 2.0 API experience:

- **Expand scope beyond fee-for-service:** Blue Button should serve all Medicare beneficiaries, not just FFS. Incorporating MA encounter data, supplemental benefit rules, and plan logic would level the playing field.
- **Unify under CMS leadership:** Rather than expanding ONC oversight or building TEFCA dependencies, CMS should directly steward Blue Button's expansion to include real-time eligibility, network, and prior auth logic.
- **Enable real-time updates:** Add webhooks or notifications to alert apps when claims, eligibility status, or coverage phase changes, which will reduce latency and enable proactive care.
- **Simplify consent & delegation:** Support OAuth 2.0 and proxy models so caregivers can access Blue Button data without repeated credentialing hurdles.
- **Standardize plan logic & rules:** CMS should publish machine-readable plan rules for coverage decisions (step therapy, PA, exclusions) via the API.
- **Allow app-based correction of errors:** Enable reporting or flagging of suspected fraud or claim anomalies directly through consumer apps.
- **Leverage public-private research partnerships:** CMS should enable public-private collaborations focused on health equity, navigation, and digital engagement. Partnerships with academic institutions and trusted digital health platforms can help validate how enriched Blue Button data drives better outcomes and supports CMS modernization goals.

Non-CMS data that should be integrated

- **Immunization registries (via CDC/state IIS):** Enables better tracking of vaccine history across systems, which is key for seniors and public health.
- **Pharmacy data from PBMs or retailers:** Offers real-time medication fill and adherence data, which is timelier than claims and can flag risk early.
- **Social support program participation:** AskMD could surface Medicaid waiver services, SNAP eligibility, or other support resources to prevent unnecessary ED use or missed care.
- **VA and DOD data:** Dual users (especially veterans) often receive fragmented care. API-based access to these records reduces duplication and adverse events.

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Sharecare's AskMD is built to solve precisely the inefficiencies Blue Button 2.0 has not yet addressed. It serves as a real-time, consumer-facing intelligence layer that uses both CMS and non-CMS data to guide patients with precision and accountability. Capabilities include:

- Real-time benefit logic from MA plans, including prior authorization and supplemental services
- Automated eligibility checks for LIS, dual status, and enrollment phases
- Provider finder tools that integrate plan directories with scheduling availability
- Fraud risk detection via pattern recognition in claims behavior or benefit usage
- Proxy workflows that simplify caregiver access while maintaining security
- Integration of SDOH and public services for whole-person navigation

AskMD doesn't require TEFCA or ONC to function but will plug into those frameworks where useful. CMS can deliver more efficient, fraud-resistant, patient-centered navigation by empowering proven tools and expanding the scope of Blue Button 2.0 under its own leadership.

PC-10

How is the Trusted Exchange Framework and Common Agreement™ (TEFCA™) currently helping to advance patient access to health information in the real world?

a) Please provide specific examples.
b) What changes would you suggest?
c) What use cases could have a significant impact if implemented through TEFCA?
d) What standards are you aware of that are currently working well to advance access and existing exchange purposes?
e) What standards are you aware of that are not currently in wide use, but could improve data access and integration?
f) Are there redundant standards, protocols, or channels that should be consolidated?
g) Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

TEFCA was designed to simplify and scale nationwide health information exchange (HIE), giving patients real-time access to longitudinal clinical data regardless of where care occurred. CMS now faces the challenge of determining whether to double down on TEFCA, reframe it for better outcomes, or pursue hybrid alternatives that deliver interoperability while reducing federal redundancy and preserving the commercial HIE ecosystem.

a) How TEFCA is helping today:

- TEFCA has created a technical and policy framework that enables secure, standardized data exchange across networks. TEFCA's emphasis on IAL2 identity proofing, FHIR compatibility, and consent-based sharing aligns well with AskMD's design and use cases.

- However, TEFCA's real-world impact remains limited. TEFCA's approach to consent, emphasizing "meaningful choice," has not reconciled fundamental policy differences across states. The varying consent policies—opt-in, opt-out, or combinations thereof—can hinder the sharing of important clinical data across state lines, affecting the scalability and effectiveness of the framework. Addressing ambiguity and closing loopholes is key to ensuring patients have clear, understandable control over their Protected Health Information (PHI).

b) Recommended changes:

- **Ensure patient-centricity:** Address regulatory ambiguities and gaps where PHI may fall outside HIPAA's direct protection, particularly when shared with third-party applications or commercial entities. Data should be automatically protected unless a patient explicitly chooses to share it, thereby minimizing risks of unintended dissemination and avoiding scenarios where a patient's health information may be used inadvertently or with unintended discrimination.
- Expand technical guidance on integration with SMART or FHIR & OAuth 2.0: This would make TEFCA plug-and-play for the digital health ecosystem.

c) **Ensure reimbursement pathways sustain existing data platforms:** CMS should fund or incentivize value-added services provided by HIEs and commercial data aggregators rather than displacing them. TEFCA should serve as the "pipes," not the entire plumbing system.

d) **Evaluate governance structures:** CMS should reassess whether TEFCA oversight belongs solely under ONC. A CMS-led governance structure could ensure stronger alignment with reimbursement strategy, coverage policy, and beneficiary access metrics.

e) Standards currently working well:

- **FHIR R4, USCDI, and OAuth 2.0:** foundational and functional for real-time, secure, interoperable data exchange
- **OpenID Connect:** supports identity verification and third-party integration

f) Standards to expand or improve:

- **CDS hooks:** Real-time clinical guidance integration
- **SMART scheduling links:** Enables user self-scheduling across provider systems
- **Price transparency APIs:** Support consumer-facing cost estimates tied to coverage rules
- **Develop clear and accessible patient consent mechanisms:** Implement clear, understandable language and user-friendly processes for real-time patient authorization of data use, including a simple and accessible mechanism for revoking consent.

PC-11

How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

b) How valuable, available, and accurate do you find the data they share to be?
c) What changes would you suggest?
d) Are there particular examples of high-performing HIE models that you believe should be propagated across markets?
e) What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

HIEs can offer a proven, regional foundation for interoperability; however, their value to patients and digital health applications varies widely. As CMS drives toward more efficient, cost-effective systems for data access, care navigation, and beneficiary engagement, HIEs represent an immediate opportunity to scale progress.

b) How valuable, available, and accurate is HIE-shared data?
In high-functioning HIEs (e.g., CRISP, Manifest MedEx, Healthix), the data is rich, longitudinal, and highly actionable—covering ED visits, inpatient records, lab results, and discharge summaries. These networks facilitate care continuity across fragmented providers, and they provide an essential source of real-time clinical signals for digital platforms. However, across the country:

- Data completeness and freshness are inconsistent.
- Batch uploads still dominate in lower-performing HIEs.
- Most do not offer consumer APIs or support for third-party patient-facing tools.
- Access rights for proxies, caregivers, and AI agents are unclear or absent.

c) Suggested improvements to support CMS goals:

- **Standardize consumer access:** Require HIEs to offer FHIR-based APIs that enable digital platforms to access clinical data with patient consent. Proxy and caregiver access should be uniformly supported.
- **Incentivize real-time data exchange:** Move away from retrospective batch processing. CMS should link HIE funding to real-time data availability and patient-facing access features.
- **Incorporate broader data types:** Encourage integration of payor claims, medication fill history, and community-based resources (e.g., food insecurity programs or transportation services) into HIE feeds.
- **Promote financial sustainability:** CMS could tie supplemental payments or Medicaid waiver incentives to HIE performance and patient access benchmarks, ensuring their sustainability without permanent federal grants.

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d) High-performing HIE models:
- **CRISP (MD/DC):** Real-time alerts for ED encounters, robust consent architecture, and multi-payor data feeds.
- **Manifest MedEx (CA):** Aggregates clinical and claims data, pharmacy histories, and analytics.
- **Healthix (NY):** Consent-driven access across >8,000 orgs, including behavioral health, with strong patient-matching tech.

e) Ongoing role of HIEs in a TEFCA-enabled (or hybrid) future:

HIEs remain indispensable regardless of TEFCA's trajectory. They can:
- **Serve as data normalizers:** Standardize inputs from hospitals, labs, and public health.
- **Support community-level interventions:** Identify regional care gaps or resource shortages for CMS pilots.
- **Fill gaps in national infrastructure:** TEFCA may never fully incorporate Medicaid, behavioral health, or SDOH sources; however, HIEs already do.
- **Act as regional on-ramps:** Help small providers and safety net clinics connect into the broader ecosystem.

Sharecare's AskMD is designed to ingest HIE data, enrich it with other sources (e.g., payor claims, user-reported info), and deliver real-time, context-aware guidance to beneficiaries. Where TEFCA or direct APIs fall short, HIEs can serve as regional backbones that AskMD translates into action. AskMD's capabilities include:
- **Data normalization across systems:** Converts fragmented HIE records into structured inputs for personalized navigation.
- **Agentic AI routing:** Ensures that lab results, discharge summaries, and medication changes trigger next steps, like provider lookups or telehealth scheduling.
- **Cross-source data fusion:** Combines HIE data with payor feeds, user-reported info, and local resource directories to create full care maps.
- **Caregiver proxy access:** AskMD supports delegated access workflows that many HIEs don't yet provide natively.

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PC-12

What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

a) How valuable, available, and accurate do you find the data they share to be?
b) What changes would you suggest?
c) Are there particular examples of high-performing HIE models that you believe should be propagated across markets?
d) What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

Patients and caregivers are burdened by fragmented, inaccessible, and confusing operational health data. This includes unclear costs, outdated provider directories, and opaque quality metrics, all of which obstruct timely care, drive avoidable costs, and reduce trust in the system. CMS seeks to identify which operational data use cases have the greatest potential to empower beneficiaries, streamline access, and reduce administrative inefficiencies.

a) Most valuable operational use cases
   - **Binding cost estimates across episodes of care:** Patients need upfront, plan-specific estimates that span all components of an episode, including facility fees, specialist consults, imaging, labs, and ancillaries, to avoid surprise billing and understand tradeoffs.
   - **Real-time provider schedule visibility:** AskMD is integrating SMART Scheduling Links and other appointment APIs to allow users to view and book next-available appointments across in-network providers.
   - **Third-party app support for appointment management:** AskMD enables beneficiaries to request, confirm, and adjust appointments from one interface, reducing phone tag, no-shows, and frustration.
   - **CMS-aligned quality and equity metrics:** AskMD can surface provider performance filtered by condition, demographics, and equity factors (e.g., language access, maternal health disparities), enabling users to make informed choices beyond proximity alone.
   - **Personalized provider matching:** AskMD uses AI to match users with providers based on benefit eligibility, condition-specific need, cultural preferences, and availability.
   - **'Verified Insurance ID' integration:** AskMD is equipped to incorporate a verified digital or physical ID card tied to plan enrollment, enabling instant eligibility checks, benefits adjudication, and accurate routing.

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b) What's possible today

Sharecare's AskMD already supports:

- Real-time eligibility verification and cost estimation through payor APIs.
- Provider lookup and filtering by plan network and availability.
- Navigation tied to plan-specific logic, including prior auth triggers.
- Appointment requests and reminders in supported systems.
- Cross-EHR scheduling integration: Unified appointment access across primary care, specialists, and virtual channels, regardless of EHR vendor.
- Automated benefit-aware navigation: Tools that dynamically adapt to plan rules, coverage exceptions, and out-of-network risks.
- Risk-adjusted quality comparisons: AskMD can surface contextualized comparisons (e.g., "highest-rated cardiologist within 10 miles who treats women over 65 on MA plan X").
- Delegated access management: Caregivers should be able to manage appointments, receive updates, and flag needs without duplicating beneficiary login processes.

AskMD is a scalable, agentic navigation platform purpose-built for these operational use cases. It offers:

- **'Verified Insurance ID' integration:** Enabling eligibility confirmation, plan validation, and secure data access across services and tools.
- **Plan-specific navigation and cost insights:** AskMD links payor APIs and network logic to real-time scheduling and benefit coordination, ensuring accurate, no-surprise care navigation.
- **Cross-platform provider access:** Integrates data from scheduling vendors, direct EHR APIs, and Care Compare to offer best-fit provider options filtered by plan, quality, and patient need.
- **Personalized operational guidance:** AI agents automate appointment reminders, benefit summaries, and follow-up tasks, reducing administrative burden for users and caregivers alike.
- **Proxy and delegated support:** AskMD enables caregivers to act on behalf of the beneficiary through authenticated pathways, which is critical for seniors with digital or cognitive barriers.

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Information blocking and digital identity

PC-13
How can CMS encourage patients and caregivers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would increasing reporting of complaints advance or negatively impact data exchange?

While the 21st Century Cures Act prohibits information blocking, the enforcement process remains underutilized by patients and caregivers. Lack of awareness, confusing terminology, and burdensome reporting systems deter participation. CMS seeks to understand how to increase the volume and quality of complaints submitted to ONC's Information Blocking Portal, and whether this would support or hinder nationwide data exchange efforts.

How CMS can encourage complaints

- **Embed awareness into routine CMS communications:** CMS should integrate information blocking education into high-visibility channels such as Medicare and Medicaid enrollment packets, MyMedicare.gov, Explanation of Benefits (EOBs), and CMS-authored emails or texts.
- **Enable point-of-friction reporting via Sharecare's AskMD:** AskMD can detect when a user encounters a missing lab result, unresponsive system, or blocked API. In these moments, a real-time prompt could explain the issue and initiate a simplified reporting workflow.
- **Simplify and streamline the reporting workflow:** The ONC portal should support API submissions from third-party tools like AskMD. Pre-populated complaint forms with system metadata and timestamps reduce errors and increase the usefulness of submissions.
- **Coordinate with advocacy and provider groups:** Partnerships with AARP, Alzheimer's Association, and care coordination entities can help educate beneficiaries and normalize complaint filing as part of health system accountability.

Impact of increased reporting

- When evaluated with context, increased complaint volume strengthens enforcement, deters bad behavior, and reveals root-cause system bottlenecks (e.g., noncompliant EHRs, payor-side denials, missing consent pathways). However, CMS must mitigate unintended consequences:
- **Avoid penalizing good-faith actors:** Define clear thresholds for what constitutes "blocking" versus technical failure.
- **Prevent overload:** Pair ONC with scalable triage tools or AI-based filtering (potentially through CMS's enterprise AI strategies).
- **Incentivize compliance:** Create reimbursement-linked incentives for provider organizations to implement consumer-friendly data access.

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Would increased complaints help or hurt data exchange?

- Increased reporting would help, as long as CMS ensures:
- Complaints are contextualized and analyzed, not just logged.
- Enforcement is remedial, not solely punitive.
- Data is used to drive policy refinement, not to create fear.

Sharecare's AskMD bridges the gap between inaccessible systems and actionable resolution:

- **Real-time friction monitoring:** AskMD detects when data is not returned whether due to denial, missing consent, or technical error, and explains what happened in plain language.
- **Complaint initiation at point-of-failure:** Users can generate a structured complaint prefilled with encounter data, identity verification, and provider/system details. This enhances report quality and traceability.
- **'Verified Insurance ID' and audit trails:** AskMD's secure ID tools ensure that complaints can be verified and linked to legitimate beneficiary identities, reducing spam or mistaken filings.
- **Embedded patient rights education:** Instead of linking users to a static ONC page, AskMD delivers tailored educational nudges that explain rights and options, contextualized to what's missing.

We believe and encourage patients and their care givers to submit information blocking complaints; however, we have seen where the methodology for complaints are also abused by 3rd parties. We highly recommend that when reviewing information blocking complaints that there is a clear process in understanding the truth source of the complaint, i.e. either the patient or the 3rd party.

PC-14

Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 credentialing service providers (CSP)):

a) What are the challenges today in getting patients/caregivers to sign up and use digital identity credentials?
b) What could be the benefits to patients/caregivers if digital identity credentials were more widely used?
c) What are the potential downsides?
d) How would encouraging the use of CSPs improve access to health information?
e) What role should CMS/payors, providers, and app developers have in driving adoption?
f) How can CMS encourage patients to get digital identity credentials?

Without a reliable and trusted mechanism for digital identity verification, patients and caregivers face fragmented, insecure, and inefficient access pathways to their health information. CMS seeks input on how digital identity credentials (e.g., CLEAR, Login.gov, ID.me) could be expanded across health systems to improve data access while accounting for the operational, legal, equity, and adoption risks that such a shift might entail.

a) Challenges in getting patients/caregivers to use digital credentials
   - **Low trust and privacy concerns:** Some users (especially Medicare beneficiaries) are hesitant to share personal or biometric data with the government or third parties. The idea of a "national digital ID" triggers concerns about surveillance or centralized data misuse.
   - **Biometric variability:** Laws like Illinois' Biometric Information Privacy Act (BIPA) severely restrict how biometric data (e.g., facial scans) can be collected or stored. This patchwork regulatory environment complicates nationwide rollout.
   - **Digital literacy and device access:** Many seniors or low-income patients lack smartphones, internet, or comfort with multi-factor authentication workflows.
   - **Fragmented ecosystem:** Patients may need different logins across provider portals, payors, and apps, which will lead to frustration and abandonment.

b) Benefits of more widely used digital identity credentials
   - Widely adopted digital identity credentials would allow patients and caregivers to securely access health records, insurance information, and care tools across platforms without needing to re-enter the same information or manage multiple logins. A universal digital identity acts like a "healthcare passport" enabling people to prove who they are once and use that credential to navigate Medicare, Medicaid, or commercial systems. This would reduce paperwork, prevent delays in accessing care, and simplify coordination for caregivers acting on someone else's behalf. It also strengthens privacy and trust by ensuring that only authorized users can see or act on sensitive health information.

c) Potential downsides
   - **Equity and access gaps:** Credentialing should not become a barrier. Making it a condition of access would disproportionately impact the same populations CMS is trying to support (older adults, rural residents, and non-English speakers).
   - **Over-reliance on private vendors:** Concentrating this function in companies like ID.me or CLEAR may create vendor lock-in, especially if their models are not fully transparent.
   - **Legal and cultural mistrust:** Some users view digital credentials as an unnecessary intrusion or step toward government surveillance, particularly when bundled with biometric components.

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d) How encouraging use of CSPs could improve data access
- **Interoperability across systems:** Digital credentials allow platforms like AskMD to connect health data across patients, payors, and care teams without repeated verification.
- **Consent management and auditability:** Identity-linked consent enables more granular, accountable data sharing, especially for caregivers, caregivers, and delegated agents.
- **Scalable navigation infrastructure:** A robust digital identity layer is essential for AskMD's real-time orchestration of coverage checks, provider search, and benefit routing.

e) Roles of CMS, payors, providers, and developers
- CMS should:
  - Establish federal baseline standards (e.g., NIST 800-63-3).
  - Subsidize credentialing costs for low-income or digitally underserved beneficiaries.
  - Ensure biometric use is opt-in and jurisdiction-compliant.
  - Include non-biometric pathways for verification.
- Payors must:
  - Accept certified credentials as part of data exchange workflows.
  - Make plan logic and coverage data available upon verified access.
- Providers should:
  - Accept federated logins to reduce redundancy.
  - Offer education on how identity credentials can improve access.
- App developers (like AskMD) must:
  - Offer credential-agnostic integration, not tied to one vendor.
  - Embed guidance, translation, and step-by-step support into onboarding.
  - Respect state-by-state biometric policies and allow opt-outs.

f) How CMS can encourage adoption
- **Voluntary-first approach:** Avoid mandates. Instead, emphasize how credentialing unlocks services, shortens wait times, and improves outcomes.
- **Transparent data use policies:** Clearly explain what data is collected, how it is stored, and who can access it. Include tools for revoking or adjusting permissions.
- **Incentivized demonstrations:** Launch public-private pilots in Medicare and Medicaid settings to test adoption, equity outcomes, and navigation enhancements.
- **Integration into Medicare, Medicaid, and Exchange touchpoints:** Credentialing could be offered during enrollment, at wellness visits, or embedded in EOBs.

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Digital health apps

PR-1

What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

a) What are the current obstacles?
b) What information should providers share with patients when using digital products in the provision of their care?
c) What responsibilities do providers have when recommending use of a digital product by a patient?

Providers, especially those in rural or underserved areas, often underutilize digital health tools due to infrastructure, incentive, and integration gaps. CMS is seeking ways to encourage broader, more effective use of approved digital products to improve care access, reduce inefficiencies, and strengthen outcomes.

Current obstacles:

- **Fragmented data access:** Providers often lack visibility into eligibility, benefit design, and patient history at the point of care, which limits their ability to prescribe or refer digital tools that align with patients' financial and clinical needs.
- **Disjointed technology ecosystems:** Most digital products do not integrate seamlessly into clinical workflows, creating duplicative work and administrative burden, especially in environments where EHR systems are outdated or unsupported.
- **Digital literacy gaps:** Rural and high-risk populations may face challenges using digital tools due to low digital literacy, language barriers, or limited access to broadband and smartphones.
- **Fee-for-service misalignment:** Current payment models often reward volume of in-person services over value-driven outcomes, leaving digital solutions under-incentivized or unreimbursed.

What providers should share with patients:

- **Cost and coverage details:** Whether the digital product is covered by the patient's plan, and if not, what out-of-pocket costs may apply.
- **Expected benefits:** What the tool is intended to do (e.g., manage diabetes, schedule visits, monitor symptoms) and how it supports the broader care plan.
- **Privacy and data use:** How patient data will be collected, stored, and used, especially if the tool is from a third-party developer.
- **Alternatives and accessibility:** Whether alternative, non-digital methods of engagement exist for patients with accessibility or language needs.

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Provider responsibilities when recommending a digital product:

- **Ensure clinical appropriateness and equity:** Providers must consider the patient's overall condition, digital access, and literacy to avoid inadvertently widening care disparities.
- **Enable seamless follow-up:** Recommend tools that integrate with care coordination systems and allow for real-time monitoring, alerts, or referrals.
- **Support ongoing use:** Providers should have access to patient-reported data and outcome signals to ensure the digital tool is achieving its intended value.

Sharecare's AskMD supports providers and patients with a unified, AI-driven care navigation infrastructure:

- **Universal eligibility verification:** A core innovation of AskMD is a 'Verified Insurance ID', which is a digital credential that not only displays insurance information but also verifies eligibility, incorporated medical history, retrieves plan details, and dynamically surfaces cost-sharing obligations. Beyond benefits integration, this feature can serve as a critical component of a federated digital identity framework, aligning with CMS's and ONC's priorities around patient authentication and frictionless data access. Designated to integrate with third-party ID solutions like biometric platforms (Amazon One, Clear) or OIDC-compliant login providers, AskMD's credential layer could help eliminate the need for manual intake and clipboard-based workflows.
- **Multimodal integration with EHRs and plan data:** AskMD integrates FHIR-based clinical data, SDOH measures (via Sharecare's Community Well-Being Index), and patient-reported insights (via RealAge and digital screeners) to give providers a 360° view.
- **AI-powered agentic support:** AskMD's agentic AI identifies the best-fit interventions (whether digital or human) for the patient, and supports task automation like appointment reminders, authorization checks, and care navigation steps.
- **Accessibility-first design**: The platform incorporates multi-language support, proxy access, and alternative pathways for users with disabilities or low literacy.

Recommended CMS actions to support provider engagement:

- **Make telehealth permanency clear:** Extend telehealth coverage beyond 2025 and support parity for audio-only visits to ensure rural provider sustainability and continued access between in-office visits.
- **Advance reimbursement for digital therapeutics:** Expand 2025 reimbursement categories to include condition-specific digital tools (e.g., for MSK, diabetes prevention, mental health), tied to verified eligibility to ensure affordability and access.
- **Fund connectivity infrastructure**: Continue support for rural broadband, device access programs, and shared community digital health hubs in FQHCs and rural hospitals.

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PR-3

How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

a) Current challenges in accessing different data formats.
b) Impact on patient care quality.
c) Technical barriers to full data accessibility.
d) Cost or privacy implications of making all data formats interoperable.

Priority level compared to other interoperability needs.

Despite decades of investment in EHR systems, much of the data stored within them remains siloed, inconsistently formatted, or inaccessible to third-party tools, providers, and patients.

a) Current challenges in accessing different data formats
   - **Unstructured or semi-structured data:** Critical patient information including diagnoses, imaging findings, or care plans often exists only in scanned PDFs, clinical notes, or attachments that are not machine-readable or easily integrated into decision-support systems.
   - **Inconsistent formatting and storage:** Key data types like lab results or test orders may be stored outside the core EHR modules, requiring manual retrieval or redundant data entry by staff.
   - **Poor integration of patient-generated data:** Symptom logs, wearable data, or caregiver notes rarely flow into the EHR in a structured or retrievable format, limiting their utility for clinical decision-making.
   - **Multichannel communication gaps:** Data shared via secure messaging, phone, or fax is often excluded from the longitudinal patient record, creating blind spots and redundancies.
   - **Limited EHR capabilities:** Many systems lack the NLP, OCR, or cross-document indexing capabilities needed to make unstructured content actionable for search, alerts, or analytics.

b) Impact on patient care quality
   - When providers cannot access or act on full-spectrum health data, it leads to:
     - Redundant testing and imaging
     - Delayed or missed diagnoses
     - Unnecessary referrals and costs
     - Medication errors from incomplete history
     - Missed care gaps (e.g., screenings, follow-ups)
     - Underutilization of risk stratification and care management
     - Ineffective care transitions
     - Lower patient trust and engagement

c) Technical barriers to full data accessibility

- **Fragmentation across systems:** EHRs, lab portals, radiology systems, and payor databases rarely share data through unified architectures.
- **Limited adoption of NLP/OCR:** Most systems lack tools to extract and standardize data from scanned documents or narrative notes.
- **Real-time data gaps:** Many providers, especially in rural or low-resource settings, operate on systems that don't support real-time data exchange (e.g., ADT feeds, prior auth status).
- **Disjointed clinical and admin systems:** Claims, coverage, and benefits data often reside outside EHR systems and are not tied to clinical workflows.
- **Delegated access and consent management:** Providers lack infrastructure to grant access to caregivers, digital tools, or proxy systems in a streamlined, privacy-compliant manner.

d) Cost and privacy implications of making all data interoperable

- Cost considerations:
    - **Infrastructure modernization**: Upgrading legacy systems to support searchable, exchangeable data formats would require investments in data storage, interface development, and OCR/NLP capabilities.
    - **Operational burden:** Especially for small or rural providers, these costs may be prohibitive without CMS support, grants, or technical assistance.
    - **Avoided costs:** Long-term savings include reduced duplicative testing, fewer adverse events, streamlined pre-visit preparation, and automation of documentation and billing workflows.
- Privacy considerations:
    - Scope of access: Full data interoperability must be governed by user-controlled consent, especially when exposing sensitive information embedded in notes (e.g., behavioral health or genetic data).
    - Regarding digital apps or other forms of third-party access, Sharecare, has become increasingly concerned about the erosion of patient privacy protections through disclosure of more information than necessary to commercial third-parties who are not always representing the patient's health care interests. Third-party access has typically been directed through HIPAA-compliant patient authorizations that are required to include reminders to the patient of their privacy rights. However, a growing number of requests are coming via patient directives. These directives, while designed to allow patients to have easier access to their medical records, are not required to include a listing of patient rights. As such, many nefarious actors will utilize this type of request, often masquerading as the patient or the patient's representative, in order to obtain PHI. Since most of these commercial requestors are not currently held to the privacy and security rules under HIPAA, this means a patient's protected health information is moving outside of any existing privacy protections. Once

outside HIPAA's umbrella, a patient's protected health information could be sold, stored on unsecure servers, or otherwise put at risk without the patient's knowledge.

- Encouraging the use of credentialing service providers (CSPs) could improve access to health information more securely. In our experience, most patients are unaware that a third-party has used a patient directive to request their PHI or what the potential implications for their PHI may be. In particular, these patients do not understand that their data is not protected in the same manner as it was in the hands of their health care provider. They are often unfamiliar with the actual commercial entity requesting the record on their behalf. This unsettles the patient and creates an added layer of work for the Covered Entities and the ROI industry as we must verify patient intention. The use of CSPs will verify and ensure the credibility of these third-party apps, allowing patients easier and safe access to their health information.
- In the context of data exchange, our members strongly support health information interoperability and access, with a focus on privacy. With new opportunities emerging with application programming interfaces (APIs) providing access to the entirety of a patient's EHI, Sharecare believes patients should have modern tools and mechanisms to access their health information. Sharecare is committed to ensuring that access to PHI is available where appropriate yet strictly limited to authorized use cases to prevent inappropriate access and maintain data security and compliance.
- In practice and as mentioned previously, PHI is sometimes shared with entities that are not directly involved in delivering or paying for patient care, and those entities do not have the same obligations under federal law to safeguard PHI. While many data requesters are authorized to receive PHI for a variety of legitimate use cases, our member organizations have seen an increase in fraudulent record requests, sometimes without patients' awareness or permission. We are concerned about heightened risks to sensitive patient data in an increasingly data-rich and digital environment. Data breaches have been on the rise for the past nine years, with an upward trend in the number of compromised medical records. In 2023 alone, 725 data breaches involving 500 or more records were reported to the U.S. Department of Health and Human Services' Office for Civil Rights, impacting over 133 million records. As access to health records continues to evolve, in the absence of a modernized patient privacy and security regime, we are concerned that the frequency of data breaches will also increase, leading to a rise in fraud across the health care industry, including programs like Medicare, Medicaid, and the Exchange.
- Granular permissions: Infrastructure must allow patients to control access at the data-type or stakeholder level, ensuring trust and legal compliance.

- Audit trails and attribution: Systems like AskMD use secure audit trails to ensure all data access and use is transparent and attributable.

e) Priority level compared to other interoperability needs

- While expanding interoperability across all formats is critical to long-term transformation, it may be best staged after foundational standards (like FHIR-based APIs for clinical and admin data, prior auth, and coverage verification) are fully implemented.
- Strategic recommendation: CMS should advance a tiered roadmap that first enforces real-time FHIR interoperability across plans and providers, while incentivizing gradual integration of structured and unstructured data formats using NLP, OCR, and AI tools.
- AskMD was architected to overcome these exact barriers through its agentic AI infrastructure and commitment to standards-based interoperability:
- Data normalization and contextualization: AskMD uses OCR and NLP to extract insights from unstructured documents (e.g., scanned records, PDFs) and converts them into structured, actionable information.
- Orchestrated navigation across formats: AskMD ingests FHIR-based EHR data, claims data, SDOH inputs, and patient-reported data connecting disparate data types into a single, contextualized experience for both patients and providers.
- Proxy and delegated access: AskMD supports patient-authorized caregiver and third-party access, ensuring broader data use while preserving control and security.
- Privacy-by-design architecture: The platform incorporates granular consent layers, biometric and credential-based identity verification, and secure access logs to maintain transparency and compliance.

PR-4

What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

Patients and caregivers face delays and friction when trying to engage in routine care tasks such as completing intake forms, viewing provider information, or managing appointments. These challenges are exacerbated by the lack of standardization, fragmented permissions frameworks, and closed scheduling systems, which limit third-party app functionality and increase administrative burden for providers and patients alike.

Needed capabilities:

- **Unified digital identity and permissions framework:** A single identity credential (e.g., CLEAR, Login.gov) integrated with user-controlled data-sharing preferences is essential to unlocking seamless third-party app access.
- **Standardized taxonomy and interoperable APIs:** CMS should mandate support for FHIR-based scheduling, intake, and directory APIs using consistent metadata standards (e.g., NPI, specialty codes, service availability windows).
- **Open scheduling and availability infrastructure:** Real-time, read-write scheduling APIs should allow trusted third-party tools to initiate, modify, and cancel appointments across EHR systems.

AskMD embeds these capabilities directly into its orchestration layer:

- Uses federated identity to verify users and link to their health data across provider and payor portals.
- Prepopulates intake forms based on insurance, prior claims, health history, and user-provided data.
- Interfaces with provider scheduling APIs (where available) to surface next-available appointments and book directly.
- Sends automated, contextual reminders to reduce no-shows and enhance appointment adherence.

Policy recommendations:

- Require FHIR-based appointment and intake APIs for providers.
- Promote or mandate acceptance of federated credentials across provider systems.
- Develop shared consent infrastructure to standardize patient-authorized third-party access.

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2. Data exchange

PR-5

Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

a) Patient access API
b) Standardized API for patient and population services
c) Provider directory API
d) Provider access API
e) Payor-to-payor API
f) Prior authorization API
g) Bulk FHIR – Do you support group ID-based access filtering for population-specific queries?
h) SMART on FHIR – Do you support both EHR-launched and standalone app access?
i) What does the process for application deployment entail?
j) CDS hooks (for clinical decision support integrations)

Providers face complexity in deploying FHIR APIs across a range of use cases, including patient access, payor exchange, directories, and prior authorization.

Sharecare's AskMD and Sharecare HDS capabilities across FHIR APIs:

- **Patient access API:** Supported via AskMD's integration with MA and Medicaid APIs; enables member-initiated eligibility checks and plan-based navigation. Uses individual query models with OAuth2 user consent.
- **Standardized API for patient and population services:** In development; will enable bulk population filtering by cohort or risk profile for digital outreach and care coordination.
- **Provider directory API:** Actively integrates third-party and payor-maintained directories for accurate, plan-specific provider search.
- **Provider access API:** AskMD is equipped to support this via delegated consent, enabling providers to access shared patient data to guide care.
- **Payor-to-payor API:** Not directly used, but data fetched from partner payors supports longitudinal histories.
- **Prior authorization API:** Currently surfaces prior auth requirements; real-time API integration in progress to enable status checks and automated prior auth tracking.
- **Bulk FHIR:** AskMD supports group ID-based filtering for campaigns (e.g., diabetic patients overdue for screening). Export ingestion supports longitudinal trend analysis.

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- **SMART on FHIR:** Supports standalone app access; integration with EHR-launched workflows varies based on provider EHR capability.
- **CDS hooks:** Under evaluation; AskMD is building intelligent nudges and proactive agentic interventions that may leverage CDS hooks to surface recommendations at the point of care.

PR-6
Is TEFCA currently helping to advance provider access to health information?

a) Please provide specific examples.
b) What changes would you suggest?
c) What other options are available outside of TEFCA?
d) Are there redundant standards, protocols or channels or both that could be consolidated?

TEFCA was designed to accelerate nationwide data exchange by creating a network-of-networks under a common trust framework. While TEFCA's primary objective is to create a standardized and unified framework for health information exchange across the United States, there are several concerns, unintended consequences, and risks associated with a nationalized information exchange. As referenced previously, paramount among these concerns is our steadfast conviction that TEFCA must unequivocally remain a voluntary mechanism for data exchange, rather than evolving into a mandatory conduit. Mandating such a singular network poses an existential risk; a significant cybersecurity breach within a centralized system could precipitate a catastrophic compromise of the entire healthcare data infrastructure, endangering both patient privacy and, more critically, patient safety on a national scale. Furthermore, Sharecare is deeply concerned by the lack of explicit patient consent and comprehensive transparency regarding how their sensitive health data is utilized and reused once it enters the TEFCA network, potentially undermining patient trust and control. This apprehension is compounded by the inherent complexities and substantial financial costs associated with implementation, which, if not meticulously managed, threaten to create disparate access and exacerbate existing market imbalances, inadvertently stifling innovation and fostering anti-competitive environments to the detriment of equitable and widespread health data exchange.

Is TEFCA helping today?

- In limited ways. TEFCA provides a trusted baseline for compliance and security, which is essential for broader participation. However, many provider systems still route most data through legacy HIEs or direct connections. TEFCA's benefits are clearer in cross-network scenarios but less so in day-to-day workflows.

b) Recommended improvements:

- **Incentivize data contribution:** Tie value-based reimbursement to TEFCA participation and data quality metrics.

- **Enhance API compatibility:** Ensure TEFCA aligns with FHIR and OAuth2 standards to enable real-time, patient-initiated data access (not just provider-pull models).
- **Support local interoperability overlays:** Allow state-based TEFCA overlays that align with Medicaid agency needs, while preserving national baseline standards.
- **Require rigorous oversight of TEFCA:** CMS/ASTP/ONC must ensure strong oversight of TEFCA to prevent dominant entities from stifling competition and innovation in healthcare data exchange.
- **Ensure voluntary TEFCA participation:** TEFCA participation should remain strictly voluntary to avoid impeding innovation, creating unnecessary duplication of efforts, and ensuring responsible allocation of taxpayor resources.
- **Enhanced scrutiny on TEFCA privacy:** CMS/ASTP/ONC should apply oversight of TEFCA to ensure that all participating network entities consistently protect patient privacy and comply with HIPAA regulations.
- **Develop clear and accessible patient consent mechanisms:** Implement clear, understandable language and user-friendly processes for patient authorization of data use, including a simple and accessible mechanism for revoking consent.
- **Achieve balanced privacy and security standards:** Implement privacy and security standards that strike an optimal balance, actively encouraging beneficial uses of health data (e.g., for research) while simultaneously preventing and mitigating misuse.
- **Foster collaborative cybersecurity solutions:** Promote collaborative efforts among stakeholders to develop robust and effective cybersecurity solutions that enhance protections without imposing unintended or financially burdensome consequences on health care providers and businesses.
- **Educate patients on data sharing risks:** Implement initiatives to educate patients on the implications and potential risks of sharing their EHI with third-party applications and entities outside of HIPAA's direct oversight.

c) Alternatives to TEFCA:

- Trusted exchange via delegated patient consent through SMART on FHIR and payor APIs.
- Enhanced interoperability via national identity frameworks and universal eligibility tools (like AskMD's 'Verified Insurance ID').

PR-7
What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

CMS seeks to ensure that healthcare providers can deliver high-quality, timely, and comprehensive data to support interoperability across the digital health ecosystem. However, providers often face technology limitations, time constraints, and lack of alignment between interoperability expectations and current incentive structures.

Key capabilities and needed strategies:

- **FHIR-based, real-time interoperability infrastructure:** Providers need systems that can ingest and transmit structured data using FHIR standards across all relevant EHR, payor, and administrative systems.
- **Automated data quality prompts:** Tools that flag incomplete or inconsistent data during routine documentation can reduce downstream data quality issues.
- **Workflow-integrated data entry and extraction:** Interoperability tools must be embedded into existing provider workflows rather than requiring additional clicks or screens.
- **Proxy delegation and identity management:** Providers need easy ways to validate identity and consent across caregivers and third-party apps to ensure secure, patient-centered data exchange.
- **Quality measures that reflect operational reality:** CMS must align reporting requirements with metrics that are already tracked in risk adjustment, patient satisfaction, and quality improvement initiatives.

AskMD supports providers in delivering complete, timely, and high-quality data through:

- **Pre-visit data collection and push to EHR:** AskMD gathers structured patient data before appointments (e.g., symptoms, medication lists, digital eligibility verification) and feeds it into provider EHRs via API, reducing manual intake.
- **AI-orchestrated data checks:** The platform flags missing or inconsistent data fields during routine patient engagement, enabling providers to correct them in real time.
- **FHIR-native design with bulk and real-time capability:** AskMD supports both patient-level and bulk FHIR transactions to ensure scalability, flexibility, and compliance.
- **Data-driven feedback loops:** AskMD monitors what data gets used, missed, or causes confusion, informing systemic improvements in data quality and exchange across the ecosystem.

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PR-8

What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a) What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?
b) In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?
c) Are there requirements CMS should consider for data registries to support digital quality measurement in a more efficient manner? Are there requirements CMS should consider for data registries that would support access to real-time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

Providers currently face excessive reporting burdens when submitting clinical quality data, often requiring duplicative documentation, separate registry submissions, and inconsistent formatting across programs.

Key capabilities and needed strategies:

- **Unified quality metrics and submission pathways:** Quality metrics should be consolidated across programs (e.g., MIPS, MA, Medicaid), using a single format and submission timeline.
- **Bulk FHIR exports with CMS-driven calculation:** Instead of requiring provider organizations to compute and submit quality metrics, CMS could ingest Bulk FHIR data and calculate performance centrally.
- **Use of claims and non-EHR data:** CMS should supplement EHR data with administrative claims, lab interfaces, and digital tool-generated inputs to avoid requiring providers to attest manually.
- **Smart registries:** CMS could establish registries that both support digital quality measurement and provide real-time insights back to providers for clinical use.

AskMD as the solution:

- **Structured gap closure workflows:** AskMD proactively identifies care gaps (e.g., missed screenings, unmanaged chronic conditions) using a blend of EHR, claims, and digital interaction data.
- **Pre-visit quality flagging**: The platform surfaces quality-related action items before and during encounters, reducing provider burden at the point of care.
- **Bulk FHIR-ready architecture:** AskMD can feed standardized data sets into CMS or registry systems to support centralized quality metric calculation.

- **Digital registry integration:** AskMD can connect to registries to both supply quality data and display comparative benchmarks or performance feedback directly to providers.

3. Digital identity

PR-9

How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship?

b) What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?

c) b. How might CMS balance patient privacy with convenience and access to digital health products and services that may lead to significant improvements in health?

Patients currently manage multiple logins across healthcare systems, creating access friction and limiting the ability to aggregate and share health data effectively. Sharecare has seen various methods of fraudulent requests utilizing provider information, with a steady increase over the past few years. The major concern is that we want to ensure that this is truly the patient and/or their patient representative accessing the information.

a) What would providers need help with?

- **Technical integration support:** Many provider portals are tied to legacy login systems. CMS can offer implementation guidance and funding to integrate NIST 800-63-3 IAL2/AAL2 credential service providers (CSPs) into existing workflows.
- **Workflow and consent training:** Providers and front-office staff need simple, role-specific training to understand how to authenticate digital credentials, recognize consent delegation, and troubleshoot access issues.
- **Liability clarity:** Providers may hesitate to rely on external credentials without clear legal guidelines or CMS-backed indemnity policies that protect against misuse or fraud tied to third-party identity verification.
- **Incentive alignment:** Encourage participation by aligning digital identity acceptance with existing measures (e.g., promoting interoperability program or quality initiatives).

b) Balancing privacy, access, and convenience:
   - **Patient-directed identity use:** CMS can require that digital credentials remain under patient control and are used only with explicit, time-bound consent.
   - **Federated, credential-agnostic models:** Providers should not be required to adopt a single CSP but rather be enabled to accept any CMS-approved credential. AskMD's federated architecture supports this model.
   - **Role-based access frameworks:** CMS can promote identity-based role delegation, enabling caregivers, case managers, and other authorized parties to act on the patient's behalf through credentialed access.
   - **Emphasis on TEFCA privacy & security:** CMS/ASTP/ONC should apply oversight of TEFCA to ensure that all participating network entities consistently protect patient privacy and comply with evolving privacy and security HIPAA regulations. Robust security and identity management protocols will be critical building trust and growing participation.

Sharecare's AskMD supports credential-agnostic digital identity workflows that:

- **Integrate across providers:** Patients use one verified login to access care navigation, scheduling, benefits information, and prior authorization workflows across systems.
- **Support delegation:** Through secure proxy workflows, caregivers can act on behalf of patients using their own credentials, reducing errors and friction.
- **Embed identity into workflow:** Identity credentials fuel secure communications, eligibility verification, and real-time updates from providers, enabling proactive care.
- **Address privacy concerns:** AskMD explains credential use clearly, stores no biometric data, and honors patient-defined revocation of access.

PR-10

Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

a) What are the challenges and benefits for providers?
b) How would requiring their use improve access to health information?
c) What are the potential downsides?
d) What impact would mandatory credentials have on a nationwide provider directory?
e) How could digital identity implementation improve provider data flow?
f) Would combining FHIR addresses and identity improve data flow?

Adoption of digital identity credentials by both patients and providers remains inconsistent, slowing down interoperability, consent management, and secure data access.

a) Challenges and benefits for providers:
   - Challenges:
   - Integrating credential validation into existing EHRs and patient portals.
   - Training staff and adapting workflows to support delegated access and credential-based permissions.
   - Concerns about vendor lock-in or forced adoption of specific credential providers.
   - Benefits:
   - Simplified identity verification for intake and portal access.
   - Reduced fraud and impersonation risks.
   - Removes confusion for commercial entities acting on a provider or patient's behalf.
   - Seamless patient handoffs and more reliable data sharing, especially across systems.

b) How requiring digital credentials would improve access:
   - Enables universal patient identity across providers and apps, improving data matching and consent.
   - Reduces need for duplicative account creation, especially for patients transitioning across care settings.
   - Simplifies proxy access workflows for caregivers, especially in Medicare, Medicaid, dual-eligible, and Exchange populations.
   - Ensures the credibility of third-party apps, allowing patients easier and safe access to their health information

c) Potential downsides:
   - Exacerbation of digital divide if credentialing requires smartphones, webcams, or broadband.
   - Privacy concerns about biometric data use, especially in states with strict privacy laws.
   - Resistance from providers unwilling to overhaul portal infrastructure.

d) Impact on nationwide provider directory:
   - Digital credentials tied to verified provider identities (e.g., NPI + credential) could reduce duplications and errors in directories.
   - Would support faster onboarding of apps and more secure EHR API access.

e) Identity and data flow improvements:
   - Credential-linked APIs can support secure, rules-based data sharing across networks, especially when tied to dynamic consent logs.
   - AskMD supports identity-anchored data routing, which reduces mismatches and ensures accurate patient-provider connections.

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f) Combining FHIR addresses and identity:
   - Yes. Binding FHIR endpoints with verified digital credentials (for both patients and providers) reduces ambiguity in data exchange and strengthens auditability.

AskMD as the solution:

- **Federated credential acceptance:** AskMD supports Login.gov, ID.me, and other CSPs without forcing patients into a single pathway.
- **Dynamic identity mapping:** Credentials power secure appointment scheduling, eligibility checks, and follow-up workflows, which would all be accessible to credentialed caregivers when needed.
- **Biometric optionality:** AskMD tailors credential workflows to state regulations and patient preferences, avoiding one-size-fits-all approaches.
- **Supports nationwide data consistency:** AskMD's orchestration engine uses credentialed identity to ensure data consistency across EHRs, claims systems, and third-party apps.

PR-11

How could members of trust communities3 (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security?

Trust communities are expected to reduce provider and patient burden while enhancing security and interoperability.

Opportunities for trust community impact:

- **Enable role-based delegation:** TEFCA-participating entities can allow credentialed users to authorize caregivers and proxies, which is critical for Medicare, Medicaid, and Exchange beneficiaries and individuals with chronic or cognitive conditions.
- **API-driven credential mapping:** Trust communities can help create a federated identity layer that links verified identities to specific provider and patient endpoints for secure, rules-based access.
- **Transparent governance and oversight**: Clear documentation and oversight of identity use and trust policies can foster provider and patient confidence in secure participation.

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AskMD as the solution:

- AskMD's architecture can operate across trust communities using federated identity tokens.
- Role-based credentialing and real-time permissions are built into the AskMD orchestration layer.
- Through integration with credential service providers (e.g., Login.gov, ID.me), AskMD can issue, verify, and revoke identity access in compliance with TEFCA IAL2 requirements.

4. Information blocking

PR-12
Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

ONC's information blocking rule includes several exceptions that may unintentionally hinder patient access or market competition.

- **Reevaluate "infeasibility" exception:** This is currently too broadly interpreted. CMS and ONC should clarify when it applies and require documentation of remediation plans by providers invoking it.
- **Refine "security" exception:** Ensure this is not misused as a blanket rationale for restricting access. Security concerns should trigger specific risk assessments, not general denials.
- **Expand data format requirements:** Exceptions that allow data to be withheld due to format or system incompatibility should be phased out as FHIR and TEFCA connectivity standards mature.

Sharecare's AskMD supports real-time transparency and contextual education for patients. Exceptions should never serve as a blanket block; instead, providers should be prompted to explain and document why access was limited, and AskMD can log and report these events to CMS or ONC through API-based workflows.

PR-13

For any category of healthcare provider (as defined in 42 U.S.C. 300jj(3)), without a current information blocking disincentive established by CMS, what would be the most effective disincentive for that category of provider?

CMS needs tools to ensure providers who are not yet subject to information blocking disincentives still comply with access requirements.

Recommended disincentives:

- **Performance measure tie-ins:** Penalize noncompliance via the Promoting Interoperability and MIPS programs. Limit access to performance bonuses if information blocking complaints are substantiated.
- **Public transparency:** Publish provider-level data on access complaints and resolution timeliness to create reputational incentives.
- **Targeted payment withholds:** Implement modest temporary payment holds for providers with unresolved complaints, coupled with remediation pathways.

AskMD can support CMS with incident documentation and pre-complaint education, reducing false positives and protecting providers from unfair penalties.

PR-14

How can CMS encourage providers to submit information blocking complaints to ASTP/ONC's Information Blocking Portal? What would be the impact? Would it advance or negatively impact data exchange?

Very few providers report information blocking issues, even though they may encounter them often.

Recommendations to encourage reporting:

- **In-EHR prompting:** Providers encountering failed data pulls (e.g., FHIR queries blocked or denied) should receive an option to submit a complaint directly via EHR workflows or tools like AskMD.
- **Anonymous and assisted reporting:** Providers should be able to report through third-party tools (AskMD, EHRs, clearinghouses) without direct attribution when appropriate.
- **Integrate into quality programs:** Tie information blocking reporting to Promoting Interoperability or ACO benchmarking programs, turning passive enforcement into a quality improvement lever.

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Impact of increased reporting:

- **Positive:** Enables CMS and ONC to identify systemic barriers, repeat offenders, and opportunities for policy improvement.
- **Risks:** May strain ONC resources and raise fears of retaliation among smaller providers unless complaints are triaged, anonymized, and tied to remediation, not punishment.

E. Technology vendors, data providers, and networks

1. Ecosystem

TD-1
What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

Adoption of digital tools among Medicare beneficiaries and caregivers remains limited due to persistent fragmentation across healthcare systems and underinvestment in solutions tailored to older adults. These gaps stem from financial pressure on payors and providers, as well as outdated assumptions about technical literacy and digital readiness in this population.

CMS and eHealth data show that most Medicare beneficiaries live in communities with internet access, and nearly 8 in 10 own smartphones (up from just 2 in 10 in 2013). According to KFF, nearly half of beneficiaries used telehealth during Q2 of 2020, and platforms like Sharecare have demonstrated the ability to digitally activate one-third of Medicare membership in just the first year.

The opportunity to expand digital engagement among Medicare populations is real and proven. What's needed now is targeted investment in ecosystems that make digital participation easy, seamless, and valuable for members and caregivers alike.

Scaling such efforts nationally is challenging. Regulatory variation, funding differences, and complex infrastructure across states mean that successful deployment requires hyper-local strategies, grounded in an understanding of social risks, local market dynamics, and the organizations that already serve Medicare and dual-eligible populations.

CMS should prioritize real-world, state-based demonstration models that offer scalable infrastructure, diverse populations, and measurable outcomes, particularly those that can engage adjacent populations across Medicare, Medicaid, and commercial coverage. Georgia represents a strong opportunity for such a demonstration. According to Sharecare's Community Well-Being Index, many communities in Georgia, including large segments of the Medicare population, face barriers to care access, economic stability, and health equity, resulting in below-average scores across physical, emotional, financial, and social well-being domains.

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Sharecare's digital platform already reaches approximately 15% of Georgia's total population and is uniquely positioned through an active public-academic research partnership with the University System of Georgia. This collaborative creates a robust, low-lift testbed to evaluate AI-powered navigation tools that improve health plan comprehension, enrollment continuity, and targeted care engagement. The platform is already integrated with major provider systems for scheduling and data exchange, and connects with both state and commercial eligibility sources—enabling seamless deployment at scale.

Our platform has been purpose-built for Medicare beneficiaries and caregivers, including those with dual eligibility and multiple coverage transitions. Experience with large employers and exchange populations enables continuity of care and messaging as individuals move into Medicare. Pre-built integrations with provider workflows, record retrieval systems, and real-time benefit verification reduce friction and allow patients to access high-quality care directly. Additionally, our community-based deployments and access to ZIP code-level SDOH data allow interventions to be precisely targeted to the individuals and communities with the greatest need, advancing both equity and efficiency in CMS-aligned models.

Short-term opportunities (next 2 years):

- Activate a public-private-academic collaboration. The proposed clinical informatics and outcomes study between Sharecare and the University of Georgia will:
    - Use an AI-powered, data-driven framework to identify the optimal mix of digital, high-touch, and community-based interventions that drive member engagement, close care gaps, and improve outcomes.
    - Engage a network of public and private innovators to rapidly pilot and refine technologies that improve healthcare access, including offering structured incentives for developers, startups, and public entities to participate both at launch and over time.
    - Evaluate, both prospectively and retrospectively, the effectiveness of an AI-enabled Verified Insurance ID and intelligent plan assistant in improving enrollment, retention, and satisfaction among Georgia Access members. This population significantly overlaps with future Medicare beneficiaries and caregiver segments, providing CMS with actionable evidence from a demographically and functionally aligned group. Oversight by UGA's Institutional Review Board (IRB) will ensure methodological rigor and generate policy insights that can be applied to Medicare and other government health programs.
- Capitalize on Georgia's diverse and transitional populations. The state includes significant numbers of rural residents, aging adults, and individuals regularly cycling between Medicaid, Exchange, and Medicare, which makes it ideal for studying continuity of care, digital equity, and plan optimization tools across coverage types.

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- Align with Georgia Access exchange timing. As Georgia transitions to a state-based exchange, CMS has a timely opportunity to observe how digital decision support influences consumer behavior during a coverage redesign, producing insights that directly inform plan comparison and navigation design.
- Reduce operational burden and cost. With infrastructure already deployed and leadership endorsements secured across state programs, CMS can test Medicare-adjacent tools with minimal startup investment, shortening the learning curve and accelerating return on insight.

Long-term strategy (beyond 2 years):

- Codify flexible digital infrastructure standards that support integration across Medicare Advantage and Fee-for-Service programs, while maintaining configurability at the state or plan level. Flexible standards will also allow developers to build navigation and engagement tools once and adapt them across Medicaid, Medicare, and Marketplace, with minimal network or data silos.
- Strengthen and evolve reimbursement models that incentivize meaningful member engagement, measurable outcomes, and sustained investment across Medicare and related healthcare segments and delivery systems.
- Study and adapt proven state models, such as the OneUtah Health Collaborative, which demonstrated how states can unite technology, policy, and provider's systems around a shared innovation framework, demonstrating how cross-sector governance can accelerate responsible development. These studies will evaluate measures from patient and caregiver satisfaction, healthcare outcomes and economic impact across and within population, and reductions in fraud, waste, and above, ensuring a deployment that is seamless, effective, and less costly.
- Reward intelligent, agentic tools that empower seniors and caregivers with timely, personalized insights into plan selection, care access, and benefit use, measured by impact on cost, satisfaction, retention, and quality.

By supporting demonstrations in states like Georgia, where the same digital infrastructure already serves Medicaid, Exchange, and near-retirement populations, CMS can generate rapid, relevant insights for Medicare without starting from scratch. This layered approach accelerates impact, reduces risk, and builds the case for scalable, Medicare-specific digital tools that reflect the real-world needs of America's aging and underserved populations.

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TD-2

Regarding CMS Data, to stimulate developer interest--

a) What additional data would be most valuable if made available through CMS APIs?
b) What data sources are most valuable alongside the data available through the Blue Button 2.0 API?
c) What obstacles prevent accessing these data sources today?
d) What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

a) What additional data would be most valuable if made available through CMS APIs?
   - Real-time prior authorization status, provider availability, encounter-level utilization trends, member-level cost-sharing information, and beneficiary-level SDOH flags are all data points that are essential to reducing friction for users navigating appointments, benefits, and financial planning.

b) What data sources are most valuable alongside the data available through the Blue Button 2.0 API?
   - Pairing CMS data with state Medicaid feeds, EHR-derived quality metrics, and commercial payor care gap data would create a fuller picture of a patient's experience. Non-traditional data sources like housing stability, caregiver support status, or digital literacy indicators could enhance equity-focused workflows.

c) What obstacles prevent accessing these data sources today?
   - The most common limitations include variable data-sharing agreements across states, lack of FHIR-based infrastructure outside the CMS ecosystem, and insufficient support for patient-directed access. Many platforms are limited by incomplete implementation of API standards or unclear guidance on integrating dual-eligibility and commercial coverage streams.

d) What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?
   - CMS should prioritize APIs that reflect user-oriented workflows. These include APIs for:
   - Digital identity management to support federated credentialing across payors
   - Provider availability and scheduling
   - Eligibility and plan design information
   - Care coordination and care gap triggers

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2. Digital identity

TD-3

Regarding digital identity implementation:

a. What are the challenges and benefits?
b. How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?
c. What impact would mandatory use of the OpenID Connect identity protocol have?

Digital identity is foundational to a scalable, secure digital health ecosystem, but adoption has been hindered by usability barriers, fragmented standards, and public trust concerns.

a) Challenges and benefits:
   - Credentialing systems such as Login.gov or ID.me offer significant benefits (single sign-on, secure access, and delegated caregiver support) but they are often confusing for beneficiaries. Solutions should offer guided credentialing workflows and multilingual, in-app support. The benefits of credentialing extend to fraud prevention, secure audit trails, and data integrity, all of which underpin a successful navigation experience.

b) How would requiring digital identity credentials impact cybersecurity and data exchange?
   - Widespread use of NIST 800-63-3 IAL2/AAL2-compliant credentials would bolster system integrity, reduce impersonation risks, and enable seamless record retrieval across platforms. AskMD's architecture supports integration with multiple credentialing providers, giving CMS flexibility to accommodate evolving standards without disrupting patient workflows.

c) What impact would mandatory use of the OpenID Connect identity protocol have?

- Requiring OpenID Connect would improve authentication standardization across digital tools and facilitate more seamless exchange of identity-related attributes (e.g., caregiver relationships, consent status). AskMD already supports OpenID-compliant identity frameworks and could serve as an implementation partner in CMS-sponsored credentialing pilots.

3. Technical standards and certification

TD-4
How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

Despite the ONC's support for FHIR and open standards, many developers are still forced to work with proprietary APIs or inconsistent FHIR implementations. This undermines data liquidity and raises costs for smaller or more innovative platforms, especially those trying to operate nationally.

CMS could:

- Mandate that any certified system for Medicare Advantage, ACO REACH, or Medicaid managed care include support for key open APIs (FHIR R4+, Bulk FHIR, OAuth2).
- Tie open API use to value-based payment bonuses, especially in programs with care coordination requirements.
- Fund pilot programs where only open-standard APIs are allowed, documenting downstream reductions in administrative burden and improvement in member engagement.

TD-5
How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payors? Who should publish such a directory, and should users bear a cost?

A centralized, FHIR-enabled provider directory would enable AskMD to more easily match users with in-network, high-quality providers who meet their specific needs.

Access to the directory should be free for end-users, including patients, providers, and payors, to promote widespread adoption and utilization. However, organizations that contribute data to the directory or utilize it for commercial purposes may be subject to fees to support its maintenance and development. This model would ensure sustainability while minimizing barriers to access.

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TD-6

What unique interoperability functions does TEFCA perform?

a) What existing alternatives should be considered?
b) Are there redundant standards, protocols or channels or both that should be consolidated?

TEFCA provides a national trust framework, but it must accelerate access for patient-directed apps. Consolidating private networks and direct connections under TEFCA and aligning on FHIR-based transactions would streamline data access.

TD-7

To what degree has USCDI improved interoperability and exchange and what are its limitations?

a. Does it contain the full extent of data elements you need?
b. If not, is it because of limitations in the definition of the USCDI format or the way it is utilized?
c. If so, would adding more data elements to USCDI add value or create scoping challenges? How could such challenges be addressed?
d. Given improvements in language models, would you prefer a non-proprietary but less structured format that might improve data coverage even if it requires more processing by the receiver?

The United States Core Data for Interoperability (USCDI) offered an initial protocol for standardizing the exchange of healthcare data; however, it faces challenges across technical complexities, stakeholder resistance to change, and concerns on impacts to workflows and training, including:

- Cost and implementation challenges, including the cost of deploying interoperable solutions and finding the right interoperability solution relative to the stakeholder need and point of integration.
- Technical challenges and incompatibility that tie to multiple EHR systems in place today, barriers to interoperability, data silos, and technical complexities that require high levels of sophistication and bandwidth to adopt.
- Resistance to change and workflow challenges from change management and ability to adopt to new standards, training required for staff to deploy, and privacy concerns surfaced by specific stakeholders.
- Ongoing changes in standards, including evolutions from USCDI v1 versus v3 and subsequent iterations, which require ongoing maintenance and investment, and can create data quality and adherence issues as standards change over time.

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While the USCDI sets a critical foundation, and there have been improvements in stakeholder communication and training across versions, needed improvements to address barriers include:

- More specific protocols for EHR data formats, processing, and required updates, aligning to transparency policies while safeguarding privacy and ensuring continued compliance across regulatory and security requirements.
- Continuation of punitive measures for non-compliance, including information blocking, combined with incentives for technology vendors who leverage advanced AI and natural learning models to seamlessly integrate across and within versions, and further, apply these data to use cases that improve member experience and limit burden on provider and payor stakeholders.
- Expansion to more granular encounter context to allow digital health and otherwise stakeholders to provide full value. For example, AskMD can fill interpretive gaps using natural language models but needs full visibility into a user's care journey to make truly personalized recommendations.
- For vendors who possess the right level of sophistication in how data is ingested and processed, including human interaction combined with innovative AI- and ML-based algorithms, these groups are able to readily ingest structured and unstructured data, and to improvements above, would prefer more context and data elements for the user – and thereby more ways to support them through their healthcare journey – even if it requires additional processing and investment.
- In addition, sophisticated vendors who possess these capabilities could also serve as 3rd party intermediaries for structuring and processing data for less sophisticated parties, limiting burden on CMS to govern, train, and reinforce standards as they evolve over time.

TD-8
What are the most effective certification criteria and standards under the ONC Health IT Certification Program?

AskMD supports certifying health IT based on real-world API performance, including response latency, uptime, and completeness of data. APIs must provide access to structured clinical data, benefits details, and care gap alerts. Certification should include patient-reported data handling and caregiver proxy access. The information blocking prohibition, which prohibits practices that interfere with the access, exchange, or use of an EHI had done a good job in promoting transparency. With the regulatory backing and enforcement through the Conditions and Maintenance of Certification, requiring health IT developers to attest to non-engagement in information blocking. However, the flexibility and scalability needs to be adaptable to various healthcare setting to accommodate future technological advancements, which still maintaining the need for the program to voluntary, and not mandatory.

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TD-9

Regarding certification of health IT:

a) What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?
b) What would be the drawbacks?
c) How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient's chart (for example, faxed records, free text, discrete data)?
d) What policy changes could CMS make so providers are motivated to respond to API- based data requests with best possible coverage and quality of data?
e) How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

Despite the proliferation of APIs, many certified systems still gatekeep data, offer incomplete endpoints, or prioritize proprietary integrations. CMS seeks to shift the ecosystem toward full API transparency and consistency.

Benefits of prioritizing APIs over software functionality:

- Modular, scalable health IT environments.
- Easier integration with consumer tools like AskMD.
- Reduced dependency on legacy systems and vendor-specific workarounds.

Drawbacks:

- Higher initial investment for legacy vendors.
- Variability in API design and testing standards.
- Need for ongoing governance of endpoint quality.

Recommended policy reforms:

- Require full-chart data exposure via standardized APIs (including scanned/faxed records and free-text).
- Incentivize provider responsiveness to API requests through performance metrics or reimbursement alignment.
- Use EHR bulk data capabilities to reduce reporting burdens and fuel proactive engagement platforms.

TD-10

For EHR and other developers subject to the ONC Health IT Certification Program, what further steps should ASTP/ONC consider to implement the 21st Century Cures Act's API condition of certification (42 U.S.C. 300jj-11(c)(5)(D)(iv)) that requires a developer's APIs to allow health information to be accessed, exchanged, and used without special effort, including providing access to all data elements of a patient's electronic health record to the extent permissible under applicable privacy laws?

To fulfill the promise of the Cures Act, CMS and ONC should enforce a policy framework where certified APIs provide transparent, complete access to all permissible EHI, not just USCDI. This includes scanned documents, physician notes, free text, and structured chart data. The current "special effort" loopholes leave developers and ultimately patients dependent on inconsistent or degraded data sets, undermining the goal of frictionless access.

AskMD relies on full EHI access to synthesize, explain, and guide members through their health history and upcoming needs. Without this access, intelligent health assistants are limited to a narrow sliver of the member's reality. A requirement to expose all data elements, supported by non-discrimination enforcement and consistent formatting expectations (such as FHIR and NLP-ready text), would create a foundation for broader innovation.

TD-11

As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or "EHI export" for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a) **Should this capability be revised to specify standardized API requirements for EHI export?**
b) **Are there specific workflow aspects that could be improved?**
c) **Should CMS consider policy changes to support this capability's use?**

Although the current EHI export rule is a strong start, CMS should now move toward standardizing exports via structured FHIR bundles and associated metadata, especially for patient population-level exports. Leaving implementation choices entirely up to health IT developers has resulted in non-uniform formats, limited usability, and increased downstream transformation work for data receivers.

AskMD uses EHI exports to populate navigation tools, generate personalized health prompts, and help users build continuity across fragmented care networks. But when exports arrive in obscure or non-parsable formats or require manual review, the system falls short. CMS could support this capability's use by requiring consistent tagging, promoting optional labeling for patient readability (e.g., "Your recent cholesterol test"), and by subsidizing open-source tooling for organizations to process these exports.

4. Data exchange

TD-12
Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Yes, CMS should endorse high-quality, non-CMS data networks to expand the breadth of actionable health information available for beneficiaries. Criteria should include timeliness of updates, data provenance tracking, security certifications, and demonstrated value in real-world navigation or care delivery use cases. Additionally, networks should support open standards and enable patient-initiated queries without extra cost or burden.

AskMD, for example, benefits from incorporating non-CMS sources such as social determinants data, state immunization registries, and payor care gap alerts. This context is essential for driving personalized recommendations and identifying access barriers. CMS's endorsement and validation of such sources would give developers and payors more confidence in integrating them responsibly and efficiently.

TD-13
What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

a) What are the primary obstacles to this?
b) What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?

Full access to EHI beyond the limited USCDI elements would unlock a new wave of intelligent digital health experiences. AskMD could deliver predictive, proactive prompts based on longitudinal insights such as trends in labs, evidence of medication nonadherence, or recurrence patterns across multiple providers. These opportunities are often missed when only a curated subset of data is made available.

The main obstacles include normalization complexity, data duplication, and inconsistent metadata tagging across sources. However, these can be mitigated by sophisticated data layer tools like those used by AskMD that apply NLP, ontology mapping, and AI classification at scale.

While USCDI offers clean structure, it limits creativity and holistic interpretation. A full EHI approach allows more innovation, particularly if paired with layered access strategies (e.g., exposing raw data to developers, but summarizing for patients). CMS should enable both and fund demonstration projects that showcase the member experience and outcomes possible through this richer data ecosystem.

TD-14

Regarding networks' use of FHIR APIs:

a) How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?
b) How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?
c) Do you interconnect with other networks? Under what frameworks (for example, TEFCA, private agreements)?

Connecting to a national FHIR endpoint directory indexed by NPI and organizational identifiers would reduce complexity for platforms like AskMD. Currently, endpoint discovery requires redundant lookups, payor-specific configurations, or third-party intermediaries. A centralized, federated directory, ideally maintained under a public-private governance structure, would make real-time query routing seamless for patient-directed applications.

Establishing these connections via FHIR (g)(10) endpoints, TEFCA-based XCA protocols, or trusted frameworks can vary by geography and network. AskMD is designed to interoperate with all of them but benefits most from FHIR-first models with reciprocal access and transparent governance. Standardizing onboarding procedures, maintaining reliability scoring, and enabling users to search providers by digital accessibility (e.g., "telehealth-ready with real-time scheduling") could elevate the value of the entire network fabric.

TD-15
Regarding bulk FHIR APIs:

a) How would increased use of bulk FHIR improve use cases and data flow?
b) What are the potential disadvantages of their use?

Bulk FHIR unlocks powerful population-level engagement and precision targeting capabilities. Instead of pulling data one patient at a time, platforms like AskMD can ingest large-scale data sets to run predictive algorithms, detect emerging trends, and launch segmented outreach. For example, nudging a cohort of diabetic patients overdue for an A1C test or identifying dual-eligible individuals at risk of ED overutilization.

The tradeoffs of bulk FHIR are manageable if implementation is done right. Concerns include increased infrastructure load, risk of data over-collection, and the need for advanced security protocols. However, the ability to close care gaps at scale, reduce administrative burden on providers, and unify fragmented records vastly outweighs these concerns.

TD-16
What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

a) Do current rules encourage scalable network participation?
b) What changes would improve alignment (for example, API unification, reciprocal access)?

Point-to-point connections remain the dominant model, but they are inefficient, fragile, and unscalable. A shared infrastructure model based on reciprocal FHIR APIs, centralized registries, and TEFCA-aligned governance would streamline participation and reduce onboarding burdens for all stakeholders.

AskMD currently supports both models, but performance, reach, and innovation are all enhanced in shared frameworks. CMS can accelerate this shift by mandating FHIR compatibility, enabling reciprocal access rules, and providing funding for platforms and providers that align with a shared trust framework.

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TD-17
What are the tradeoffs of maintaining point-to-point models vs. shared network infrastructure?

Operational viability hinges on CMS's role as both infrastructure steward and market signaler. CMS can provide core funding for national networks, especially those facilitating patient access, and subsidize platform integration costs for smaller players.

CMS could also publish usage benchmarks, ROI studies, and user experience metrics to show the value of data sharing. AskMD, for example, can demonstrate reduced avoidable ED visits and higher member engagement when integrated with reliable data-sharing networks. These types of outcomes can justify long-term public investment while attracting private-sector participation.

5. Compliance

TD-18
Information blocking:
Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

a) What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?
b) Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

AskMD has encountered several forms of information blocking: EHR vendors that reject FHIR API queries citing "security risks," systems that withhold PDF reports or scan images unless requested via fax, and platforms that impose unnecessary credentialing hurdles to block patient-directed apps.

To counter this, CMS and ONC should publish transparency dashboards that report API uptime and denial rates, classify repeat offenders, and allow anonymous submission of developer-blocking behaviors. Additionally, CMS could offer safe harbor guidance for good-faith data sharing efforts to protect stakeholders from over-corrective penalties.

CMS should also clarify that all certified health IT actors, including EHRs, HINs, and payor systems, must provide data without discriminatory restrictions—especially when the request comes from a third-party app with proper authentication.

TD-19

Regarding price transparency implementation:

a) What are current shortcomings in content, format, delivery, and timeliness?
b) Which workflows would benefit most from functional price transparency?
c) What improvements would be most valuable for patients, providers, or payors, including CMS?
d) What would further motivate solution development?

Current price transparency files are technically compliant but practically unusable for most consumers. Machine-readable formats, while rich in content, lack contextual relevance. Patients don't want to parse spreadsheets. They want to know "how much will this MRI cost me, based on my plan, at the nearest facility?"

AskMD can bridge this gap by turning CMS pricing data into personalized provider comparisons that factor in deductibles, co-insurance, and quality indicators. To make this possible, CMS should:

- Mandate plan-specific cost-sharing APIs that pair with pricing data.
- Require real-time data refreshes for procedures with variable rates.
- Incentivize benefit managers and TPAs to surface net cost estimates through interoperable channels.

For developers, clearer standards and reference implementations would motivate greater investment. For patients, this would make cost an actionable factor in decision-making, not a post-care surprise.

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F. Value-based care organizations

1. Digital health adoption

VB-1
What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

ACOs and other APM participants absolutely should be utilizing digital health management and care navigation products as these tools will help them keep patients healthier and reduce avoidable costs, directly supporting shared savings and quality goals. However, adoption in these organizations has been slower than expected.

One key incentive is financial alignment. CMS could explicitly allow or even reward investment in digital health tools as part of APM operational budgets or benchmarks. CMS might permit ACOs to count the cost of providing tools such as Sharecare's AskMD to patients as an "allowable expense" that doesn't negatively impact their spending benchmark (or include it as part of care coordination payments). If ACOs know that implementing a comprehensive digital solution for all their beneficiaries could help prevent ER visits and that CMS recognizes this cost in their model, they would be more inclined to do it.

In the MSSP, CMS could incorporate use of Sharecare's AskMD as a quality improvement activity or bonus factor. For instance, add a quality measure or scoring element that tracks patient engagement via digital tools. ACOs performing well on that could receive a slight increase in shared savings.

Current obstacles preventing broader adoption in ACOs include:

- **Upfront costs and budget constraints:** ACOs operate on thin margins and lack flexibility to invest in digital tools, especially if the ROI is unclear.
- **Workflow complexity:** Digital tools often require new workflows or coordination across teams, and many ACOs lack the change management infrastructure to implement them smoothly.

- **Variable patient readiness:** especially in Medicare, Medicaid, and Exchange populations, concerns around digital literacy or tech fatigue persist, though these are often overstated and addressable.
- **Fragmented health IT ecosystems:** some ACOs span multiple EHR systems, so integrating a third-party app that needs data from all can be complicated, though technologies like FHIR are easing this
- **Uncertainty around reimbursement and compliance: Lack** of clarity about how CMS treats digital investments under MSSP benchmarks or quality metrics discourages innovation.

Policy solutions and incentive structures:

- **Make digital infrastructure a covered investment:** CMS should explicitly allow digital health and navigation tools as "allowable expenses" that do not negatively affect an ACO's benchmark. This is akin to the ACO Investment Model (AIM), where up-front funding was provided to support infrastructure enhancements. Tools like Sharecare's AskMD, which guide patients toward the right site of care, reduce unnecessary ED utilization, and support chronic disease self-management, clearly fall within the spirit of such models.
- **Performance-based rewards for engagement:** CMS could introduce performance bonus mechanisms that reward ACOs for deploying certified digital engagement platforms. For instance:
    - Add digital navigation uptake as a quality improvement activity or supplemental quality domain.
    - Reward ACOs that demonstrate increased screening adherence, medication reconciliation, or care plan follow-through via digital engagement metrics.
- Incorporate patient-reported experience measures related to navigation, education, and care confidence.
- **Dedicated per-member-per-month (PMPM) payments:** Inspired by prior CMS programs like CPC+ and Primary Care First, CMS could create a small, incremental PMPM add-on for certified digital navigation deployments. ACOs using AskMD, for example, could receive reimbursement per beneficiary per month for demonstrating active enrollment and use, especially in high-risk populations.
- **Operational integration via TEFCA and FHIR:** AskMD is designed to interoperate using FHIR APIs and CMS-certified endpoints. CMS can accelerate real-world implementation by tying digital navigation into TEFCA participation or requiring APIs to support payor and patient-facing navigation tools.

Pilot programs built around state frameworks:

- The OneUtah Digital Health Initiative offers a replicable template. By piloting a state-level framework with built-in APIs, consent structures, and engagement standards, and then allowing ACOs to connect via certified apps like AskMD, CMS can demonstrate how digital tools drive measurable outcomes in chronic care,

behavioral health, and health equity domains. A state-first approach allows CMS to validate and then scale successful models nationally. Georgia is already positioned as such a proving ground. Backed by the Georgia Insurance Commissioner, Georgia Department of Community Health Commissioner, and the President of the University of Georgia, and supported by the Georgia Governor, the state has endorsed a CMS-supported pilot to demonstrate AskMD's role in delivering AI-powered care navigation across Medicare, Medicaid, and Exchange populations. This level of stakeholder alignment provides CMS with a unique opportunity to evaluate value-based innovation in a real-world, cross-payor setting, and scale that works.

- Peer benchmarking and technical assistance: Many ACO leaders hesitate to adopt new tools simply due to uncertainty. CMS should fund or coordinate a learning collaborative focused on "Digital Engagement in ACOs", showcasing early adopter results and pairing ACOs with technology implementation partners. If an ACO sees that another organization improved HEDIS performance or reduced inpatient admissions by using a digital tool, that serves as a powerful non-financial incentive.

VB-2
How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

While APMs promote outcomes over volume, they often fail to include concrete incentives or infrastructure support for adopting AI-driven tools, real-time data platforms, or personalized digital engagement, especially those that can proactively close gaps in care. CMS has an opportunity to shift from encouraging general innovation to operationalizing specific capability domains that directly correlate with APM success. To strengthen APM outcomes and support sustained improvements in cost and quality, CMS should explicitly encourage the integration of modern technologies, such as AI-powered navigation platforms, proven clinical LLM models, advanced analytics, and health data interoperability services, into the infrastructure of value-based care arrangements. Rather than treating these capabilities as optional enhancements, CMS can embed them directly into model design, performance evaluation, and incentive structures, ensuring that innovation becomes standard practice rather than a pilot exception.

1. **AI-powered patient navigation and engagement:**
   Modern APM participants benefit significantly from tools that guide patients through the complexities of the healthcare system while closing gaps in care. Sharecare's AskMD, for example, uses conversational AI to triage symptoms, surface actionable guidance, and facilitate next steps such as scheduling appointments or verifying benefits. Unlike general-purpose chatbots, this AI is trained on medical knowledge graphs, clinical guidelines, and patient-reported symptom patterns, ensuring that its

recommendations are context-aware, aligned with clinical best practices, and continuously refined through performance monitoring. It evaluates urgency, accounts for comorbidities and medication interactions, and flags when escalation to a provider is necessary. Critically, AskMD integrates with payor data to ensure that recommendations are cost- and coverage-aware, which helps patients follow through with care more confidently and reduces delays or abandonment due to financial uncertainty.

2. **Interoperable health information access:**
   Timely access to comprehensive clinical and claims data remains a persistent challenge in APM execution. Sharecare's Health Data Services (HDS) business unit enables real-time release of information workflows across some of the nation's largest health systems. These services ensure providers and payors can retrieve complete records for documentation, risk adjustment, and care coordination.

   In value-based models (where performance often hinges on accurate coding, care gap closure, and timely intervention) such access is indispensable. CMS could require APM participants to demonstrate real-time record retrieval capabilities, and include record access infrastructure as an allowable investment in shared savings budgets.

3. **Analytics and risk stratification:**
   Beyond foundational data access, CMS should promote the use of analytics platforms that support dynamic, real-time risk stratification and predictive modeling. AskMD leverages real-world data to identify high-risk individuals and match them with timely interventions. These solutions allow APM participants to prioritize outreach and care coordination where it has the greatest ROI, such as redirecting patients from high-cost ER use to supported outpatient management.

   AskMD integrates with these insights by proactively prompting people to complete screenings, revisit care plans, refill medications, or connect with care managers. CMS should require APM entities to submit digital engagement and risk targeting strategies, and reward performance tied to appropriate outreach and documented risk mitigation, especially among high-need Medicare, Medicaid, and Exchange subgroups.

   CMS can also incentivize analytics by requiring population-level data normalization and exchange capabilities (e.g., via FHIR bulk data export, USCDI+ datasets), allowing APMs to act on whole-population insights, not just point-of-care data.

   Care coordination and closed-loop referral. One of the clearest gaps in value-based execution remains care coordination, especially referral follow-through and accountability across care silos. AskMD's triage system can connect patients directly

to in-network providers and track next-step adherence, helping APMs ensure closed-loop referrals and follow-up care.

CMS could build referral completion into quality metrics or model evaluations, especially for high-cost specialties or post-discharge care. Likewise, requiring support for interoperable, updateable care plans using FHIR CarePlan standards would move many APMs away from PDF workflows toward scalable, team-based care planning.

Responsible use of AI and emerging technologies:

- To ensure safety, equity, and transparency, CMS should establish guardrails for AI deployment in APM contexts. These should focus on:
- Algorithmic fairness and bias detection (especially for Medicare, Medicaid, and dual-eligible populations)
- Auditability and transparency in how patient-facing tools generate recommendations, distinguishing clinical vs. informational support
- Human-in-the-loop design where clinical escalation is appropriate

As AI and large language models (LLMs) become more common in consumer-facing tools, CMS can also standardize expectations for explainability, especially where tools influence decision-making related to covered services, care navigation, or benefits education.

4. **Patient-reported outcomes and digital experience:**
   CMS should more closely align APM success with patient-reported outcomes and digital engagement metrics. Sharecare's AskMD, for instance, collects structured patient-reported outcomes (PROs) and experience data (like satisfaction and symptom resolution) that can feed directly into digital quality measures (dQMs) and care dashboards.

   APMs could be encouraged or required to include PRO collection as part of their annual reporting or quality monitoring, especially for populations with chronic conditions or complex needs. CMS might also establish a digital engagement CAHPS sub-measure, tracking whether patients felt supported in understanding their care, benefits, and next steps, areas where AskMD adds measurable value.

5. **Scaling through regional frameworks like OneUtah and advancing a Georgia demonstration pilot**
   Finally, CMS should look to successful state-level digital health transformation models as proof-of-concept for national APM expansion. The OneUtah Health Collaborative provides a compelling example, aligning payors, providers, and community resources around shared digital infrastructure and engagement standards. CMS could replicate this model in priority geographies by funding multi-

payor state collaboratives, giving APM participants access to shared AI tools, common data hubs, and coordinated engagement platforms.

Georgia is well-positioned to serve as a CMS-supported demonstration pilot. Backed by a coalition of state leaders, the state is prepared to demonstrate how digital engagement tools can improve access, care coordination, and cost efficiency across Medicare, Medicaid, and Exchange populations. Building on Utah's framework, a Georgia demonstration would enable CMS to validate how aligned leadership and intelligent infrastructure can accelerate value-based outcomes and inform broader national implementation.

VB-3
What are essential health IT capabilities for value-based care arrangements?

a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.
b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

While ONC-certified EHRs provide a functional baseline, most APM participants need a more agile and interoperable digital infrastructure to manage care across fragmented networks, support timely interventions, and meet evolving quality requirements. CMS should modernize its expectations to reflect the real-world tools and data flows that determine success in value-based care.

1. **Integrated and interoperable care planning:**
   Effective value-based care hinges on shared, dynamic care plans—especially for patients with chronic or complex conditions. However, most ACOs and APM participants still rely on static PDFs, disconnected systems, or internal notes not visible across settings.

   Sharecare's AskMD addresses this by converting real-time patient interactions (e.g., symptom checkers, guided surveys) into structured summaries that inform or populate care plans. These can be integrated with FHIR CarePlan endpoints or shared with authorized providers and caregivers.

   CMS should promote adoption of standards-based, interoperable care planning tools and reward systems that can:
   - Continuously update care goals
   - Accept inputs from patient-reported sources
   - Make care plans visible to multi-provider teams

2. **Real-time event notification and data access:**
   Knowing when patients enter or exit hospitals, SNFs, or EDs is critical for timely engagement. Many APMs rely on ADT feeds from HIEs, but coverage is inconsistent. In parallel, retrospective documentation gaps due to missing records reduce HCC accuracy and delay follow-up care.

   Sharecare's AskMD, via its Health Data Services line of business, provides both:
   - Real-time ADT integration, when available, to trigger post-acute outreach
   - On-demand external chart retrieval, pulling records from across EHRs using patented technology for identity matching and consent management

3. **Normalization and analytics-ready data infrastructure:**
   Value-based care lives and dies by the data, but only if it's normalized, complete, and timely. AskMD accepts FHIR, CMS claims, and SDOH feeds and transforms them into actionable insights through the RiskAgent engine, allowing dynamic care path adjustments and real-time engagement.

   Essential health IT functions in this domain should include:
   - Automated normalization of incoming data (e.g., LOINC, SNOMED)
   - Support for real-time risk score updates
   - Stratification models that can integrate social, behavioral, and clinical data

   CMS could require APMs to maintain a centralized or federated data environment, either directly or through a partner like AskMD, that supports patient-level insight delivery for both clinical and non-clinical users.

4. **Automated quality measurement and patient attribution tools:**
   Today, quality tracking often lags behind performance, with data only available after the measurement year. Systems that capture and report eCQMs in real time (ideally tied to patient interaction) are crucial to driving mid-course correction and identifying improvement opportunities.

   CMS should encourage or require that APM participants use certified systems capable of:
   - Real-time quality scoring
   - Receiving structured patient-reported health data
   - Attribution across multiple provider touchpoints (especially critical for MSSP)

5. **Patient and caregiver engagement interfaces:**
   PMs rely on more than provider actions. They succeed when patients understand and engage with their care plan. Sharecare's AskMD's patient-facing tools deliver:
   - AI-powered symptom triage
   - Personalized, benefits-aware care guidance

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- Dynamic support for next best actions based on real-time claims, eligibility, and coverage information

CMS should promote certified or pre-approved tools that:

- Are interoperable with CMS and payor APIs
- Can personalize guidance using verified coverage data
- Track and report patient engagement (e.g., logins, prompts completed, screenings triggered)

Especially in rural or resource-limited APMs, scalable digital engagement tools like AskMD enable organizations to meet quality metrics without expanding their care management teams.

VB-4
What are the essential data types needed for successful participation in value- based care arrangements?

To deliver on the promise of value-based care, organizations must have access to a comprehensive, timely, and interoperable set of data types. These data types should span clinical, behavioral, administrative, and social domains to offer a 360-degree view of each patient's health journey. The most essential categories include:

1. **Claims and utilization data:**
   Claims data is foundational for understanding total cost of care, identifying service use patterns, and detecting high-cost events outside a provider's direct network. Monthly or quarterly feeds from CMS (for MSSP ACOs) and from MA plans are crucial to track spend, flag care fragmentation, and identify avoidable utilization such as ED visits or post-acute readmissions.

2. **Clinical data (EHR and HIE):**
   This includes diagnoses, lab results, vital signs, immunizations, imaging, problem lists, and procedure histories. Near real-time access is essential for managing chronic conditions, closing care gaps, and meeting quality measures.

3. **Social determinants of health (SDOH):**
   Capturing structured SDOH data such as housing stability, food insecurity, transportation access, and digital literacy is vital for identifying risk drivers that lie outside the clinical setting. Many providers are now integrating SDOH screening tools into EHRs, but this data must be standardized and shared.

4. **Behavioral health and substance use data:**
   Access to mental health diagnoses, behavioral screenings (e.g., PHQ-9), treatment plans, and medication history is essential but often siloed due to privacy

constraints. ACOs and other APMs need consented access to this data to coordinate care, avoid conflicting prescriptions, and understand how behavioral conditions impact physical health. Structured, shareable behavioral data should be integrated with the same urgency as physical health data.

5. **Pharmacy and medication adherence data:**
   Medication fill data (from pharmacy claims or e-prescribing systems) is critical to understand adherence, detect polypharmacy risks, and support reconciliation. Without this data, an ACO may assume a patient is controlled on therapy that was never picked up.

6. **Real-time or streaming data:**
   Dynamic data such as ADT feeds, remote monitoring (e.g., blood pressure, weight, glucose), and wearables provides critical signals between visits. These data streams allow care teams to intervene proactively, whether deploying case managers post-discharge or responding to home-monitored health metrics.

7. **Patient-reported outcomes and experience data:**
   PROMs (e.g., pain scores, mobility status, satisfaction surveys) give insight into functional outcomes and patient perspective, which is important for assessing whether interventions are improving quality of life.

8. **Master reference data (patient and provider identity):**
   Accurate matching is critical. This includes Medicare/Medicaid/Exchange IDs, payor-assigned IDs, and NPIs tied to provider directories.

   A robust value-based data infrastructure must go beyond EHR-bound clinical snapshots. It must incorporate real-world data sources like SDOH, behavioral health, pharmacy fills, remote monitoring, and patient input.

AskMD ingests and orchestrates clinical, claims, pharmacy, behavioral health, SDOH, wearable, and patient-reported data into a real-time, longitudinal profile that informs personalized triage, proactive outreach, and care navigation. Its intelligent framework ensures accurate identity matching, benefit alignment, and timely escalation across fragmented systems, enabling value-based organizations to act on the full picture, not just clinical snapshots.

2. Compliance and certification

VB-5

In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

ONC certification provides a foundational technology framework that supports value-based care by ensuring baseline capabilities such as data exchange, patient access, e-prescribing, and structured quality reporting are available across health IT systems. These certified features lay the groundwork for ACOs, PCMHs, and other APMs to operate within a shared digital infrastructure.

However, many VBC organizations must layer additional capabilities such as consumer engagement platforms and cross-provider record retrieval on top of CEHRT to realize the full potential of data-driven, coordinated care. For example, AskMD through their HDS LOB fills a critical gap by facilitating real-time chart acquisition across multiple health systems, including those outside a provider's immediate EHR ecosystem. This enhances retrospective risk accuracy, closes documentation gaps, and supports quality metrics reporting that may otherwise be delayed or incomplete.

Similarly, AskMD integrates CMS-certified APIs with patient-reported data and benefits intelligence to create a guided experience that helps patients understand their coverage, navigate care options, and stay on track with quality measures (e.g., preventive screenings, medication adherence). While not part of ONC certification, these functions directly advance value-based goals and rely on interoperability with certified systems to function.

ONC certification supports essential interoperability and documentation functions, but value-based care participants often need enhanced capabilities that build on these certified foundations to achieve VBC outcomes.

VB-6
What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

Current ONC certification criteria cover foundational interoperability and documentation standards but do not account for modern, patient-centric tools that drive engagement, cost transparency, and cross-system care coordination. As APMs scale, gaps in real-time benefits data, digital navigation, structured intake, and whole-person care support limit the effectiveness of digital solutions.

Needed capabilities:

- Real-time benefits verification & cost transparency: APM participants need technology that gives patients and providers point-of-care access to individualized coverage details, prior authorization requirements, and formulary-aligned treatment alternatives.
- Most EHRs generate referrals but don't track status or share plans dynamically across systems. In APMs, failure to confirm referral follow-through or coordinate across settings leads to missed savings and fragmented care.

- CMS should expand certification criteria to include integration of PGHD (patient-generated health data) from validated tools to reduce data entry burden and close clinical-administrative gaps.
- Analytics-ready population-level data exports: Bulk FHIR APIs are nascent and not yet widely implemented. APM success depends on analyzing population trends, quality performance, and care gaps in real time.
- Certification could require robust support for standardized population-level exports, including SDOH and behavioral indicators.

AskMD bridges certification gaps by delivering real-time benefits verification, personalized navigation, closed-loop referral tracking, and patient-generated data intake through a unified engagement layer. Its infrastructure supports FHIR-based exports, structure intake, and cost-aware guidance at the point of decision, which empowers APMs to act on whole-person data and meet value-based goals beyond current ONC-certified capabilities.

VB-7
How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

APM participants need more than basic EHR certification. They require tools for coordination, analytics, and engagement. But forcing one-size-fits-all CEHRT requirements adds burden, especially for small or rural providers. CMS and ONC must offer a modular, scalable approach that sets expectations without impeding innovation or access.

- Define core vs. modular certification paths: CMS can mandate a minimal CEHRT core, focused on interoperability, structured quality data capture, and patient access, and then allow providers to customize additional modules based on APM type.
    - For example: A rural MSSP ACO might choose a care management module but skip modules for public health surveillance not relevant to its scope.
- Recognize third-party tools that extend certified systems: Allow digital tools to fulfill required capabilities (e.g., PGHD capture, symptom triage, or care navigation) even if they operate outside the EHR.
- Tiered certification models for scale and equity: CMS and ONC could create a “CEHRT-lite” option emphasizing:
    - FHIR interoperability
    - Real-time data exchange (e.g., TEFCA participation)
    - PGHD intake
    - Patient/caregiver engagement tools

This helps smaller APMs participate in VBC without the technical debt of hospital-grade EHRs.

VB-8

How can other HHS policies supplement CEHRT requirements to better optimize the use of digital health products in APMs? As an example, requirements under the Conditions of Participation for hospitals (42 CFR 482.24(d)) require hospitals to transmit electronic patient event notifications to community providers. What barriers are in place preventing APM participants from receiving the same notifications?

Non-ONC levers like conditions of participation (CoPs) and CMS payment policies can play a vital role in reinforcing digital health adoption across APMs.

Conditions of participation for hospitals and post-acute providers:
CMS could require providers in value-based models to participate in regional HIEs or TEFCA networks, ensuring data exchange across the full continuum of care. This would allow digital navigation platforms to access a more complete clinical picture and personalize engagement more accurately.

Transition of care mandates:
CoPs could be updated to mandate electronic summary sharing (e.g., via FHIR or Direct) between hospitals, SNFs, and primary care after discharge. This enhances the ability of APM participants to deliver timely follow-up and reduces readmissions.

Payment policy Incentives for digital engagement and navigation:
CMS can expand or enhance reimbursement for remote monitoring, asynchronous digital visits, or time spent reviewing data from patient-facing platforms. Bonus incentives tied to offering beneficiaries digital navigation or symptom triage tools can also drive adoption without mandates.

Together, these policy levers can normalize the use of digital infrastructure like Sharecare's AskMD in APM delivery models, improving both experience and outcomes while lowering burden on providers.

VB-9

What technology requirements should be different for APM organizations when comparing to non-APM organizations (for example, quality reporting, and interoperability)?

Organizations participating in alternative payment models (APMs) operate in fundamentally different contexts than fee-for-service (FFS) providers. As such, they should have distinct technology expectations that reflect their greater need for interoperability, analytics, and patient engagement capabilities, but also benefit from streamlined requirements that acknowledge reduced reliance on traditional billing-centric workflows.

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APM-specific requirements should include:

- Higher standards for data exchange (e.g., FHIR API access, TEFCA participation).
- Population-level analytics capabilities, such as bulk data export and integrated risk stratification.
- Patient and caregiver-facing digital tools that promote engagement and close care gaps , which enables symptom triage, navigation, and benefit-informed care guidance tailored to Medicare, Medicaid, and Exchange populations.
- Access to cross-network medical records which improves risk score accuracy and care coordination across health systems.

Reduced requirements may include:

- Certain clinical documentation or billing features that are less relevant in shared savings or capitated models.
- Flexibility in how data is submitted for quality measurement, for example, allowing use of third-party aggregators instead of EHR-native eCQM calculations.

By aligning technical requirements with functional realities, CMS can support innovation and efficiency in APMs while reducing redundant burden.

VB-10

In the Calendar Year (CY) 2024 Physician Fee Schedule final rule (88 FR 79413), CMS established that CEHRT requirements for Advanced APMs beyond those in the "Base EHR" definition should be flexible based on what is applicable to the APM that year based on the area of clinical practice. What certification criteria should CMS identify under this flexibility for specific Advanced APMs, or for Advanced APMs in general? Are there specific flexibilities or alternatives to consider for smaller or resource-constrained (such as rural) providers in meeting CEHRT requirements without compromising quality of care or availability of performance data?

A tiered certification framework can enable flexibility for Advanced APMs without compromising on care quality or data integrity. CMS should distinguish between core certification criteria that are foundational to care coordination and measurement, and context-specific criteria that may be unnecessary for certain APM types or burdensome for smaller/rural entities.

Retain/core criteria:

- e-prescribing
- API access to patient and population data (e.g., g(10))
- Bulk data export
- Interoperability standards (e.g., FHIR, Direct)
- Structured quality measurement data capture and export

Flexible/optional criteria:

- Public health reporting that is not directly tied to the APM population.
- Patient-specific education modules (if alternate engagement platforms are used).
- Native EHR eCQM calculation, if the APM uses external platforms to aggregate and report.

Platforms like Sharecare's AskMD help meet core quality goals through patient-reported data and engagement tracking. AskMD's HDS LOB ensures EHR-neutral access to chart data, fulfilling audit and care continuity needs without mandating duplicative EHR configuration. This modular approach supports flexibility and scalability, ideal for APMs navigating diverse tech stacks.

3. Technical standards

VB-11
What specific interoperability challenges have you encountered in implementing value-based care programs?

Value-based care exposes deeper interoperability challenges than FFS models due to its emphasis on coordinated care across diverse settings. Key barriers include:

Data silos and fragmented systems:
Many ACOs operate across multiple EHRs (e.g., Epic, eCW, Cerner, homegrown systems) and aggregated provider systems. Consolidating a patient's full record is resource-intensive and often incomplete. Third party solutions mitigate this by retrieving structured records across systems and providing them to the APM entity or authorized third-party tools.

Attribution and roster inconsistencies: Patient matching inconsistencies
Reliable and timely attribution is a cornerstone of success in VBC, without unified identity management, data fails to reconcile across systems (e.g., "Robert Smith" in one, "Bob Smith Jr." in another). This creates blind spots and costly duplication in patient care leading to fragmented care management, inaccurate risk adjustment and quality outcomes. Solutions must include APIs that enable real-time roster updates, identity inconsistencies across data sources, and attribution-aware workflows Digital credentials and a consistent identity layer (as proposed in VB-14)

Lack of real-time data feeds:
Claims data can lag weeks to months. Even HIE feeds (ADT, lab results) are unreliable or

unavailable in many regions. APMs need near real-time event data to initiate timely follow-ups that provides and intersection of claims and clinical data.

Inconsistent data standards implementation:
Even with FHIR adoption, payload consistency varies (e.g., missing codes, custom extensions). CMS should enforce stricter FHIR conformance testing for certified systems. This lack of standardization undermines key workflows in value-based care, including accurate attribution, risk adjustment suspecting, and timely care gap identification. For example, if a FHIR bundle omits structured diagnosis coding or visit-level detail, suspect algorithms fail, care managers miss open gaps, and stratification models underperform.

VB-12
What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

CMS can advance both standardization and flexibility by anchoring programs in a shared technical infrastructure (e.g., USCDI, FHIR, TEFCA), while allowing APMs to customize workflows, data use, care delivery workflows and technology choices on top of those rails.

Recommended approach:

- Standardize the "pipes" (e.g., APIs, interoperability protocols) while letting APMs tailor what flows through them.
- Support modular capability adoption, such as standardized "plug-ins" for care planning, remote monitoring, or benefit verification. For example, an oncology APM might activate a FHIR-based remote monitoring module, while a primary care ACO might focus on SDOH data capture and shared care plans. This structure supports the CMS goals of higher quality and lower cost care.
- Define core outcome measures but allow varied digital pathways to meet them. For instance, CMS could specify that APMs must track medication adherence but permit it via EHR data, pharmacy claims, or through AskMD.

Sharecare's AskMD exemplifies innovation layered on top of standardized frameworks. It consumes FHIR and claims data, performs triage, and guides users through benefit-aware next steps. AskMD also supports standardized data export while enabling flexible usage, from risk score validation to gap closure documentation that integrates into population health workflows.

VB-13
What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

Value-based care models increase provider burden when technical standards do not reflect real-world workflows. Despite ONC's progress with CEHRT and USCDI, key data elements, functionality gaps, and lack of support for third-party platforms continue to hinder automation, risk scoring, and patient engagement.

To better support APMs, certification criteria must:

- Simplify and standardize care coordination documentation, including SDOH, caregiver relationships, and shared care plans, while supporting end to end risk adjustment and quality reporting in workflows.
- Support real-time digital quality measurement (dQMs) that integrate structured data from patient interactions, claims, and EHRs.
- Promote automated integration of third-party platforms, including apps like AskMD that triage patients and route structured data back into provider workflows.
- Expand support for decision support tools (e.g., CDS hooks) that guide cost-effective, evidence-based care with minimal provider effort

Sharecare's AskMD fills the functional void left by CEHRT limitations by:

- Capturing structured patient input (symptoms, medication adherence, screenings, satisfaction) and mapping them to CMS measures.
- Enabling data handoffs from patient to provider via secure APIs, reducing manual intake and documentation redundancy.
- Interfacing with payor APIs to surface real-time formulary and benefit data, which reduces abandonment and downstream costs.
- Offering embedded AI triage aligned with clinically validated protocols and escalating to live care navigators when appropriate.
- Driving patient adherence and self-reporting, enabling real-time quality tracking outside of the clinical setting.

VB-14
How could implementing digital identity credentials improve value-based care delivery and outcomes?

Lack of a unified digital identity framework hinders accurate data exchange, complicates caregiver access, and leads to misattribution or duplication in risk scoring and quality tracking. APMs are particularly vulnerable to this friction because they must track care across multiple providers/systems and they rely on clean attribution to assign financial and clinical responsibility.

CMS should support a federal identity standard that:

- Authenticates patients and caregivers across EHRs and digital tools.
- Enables proxy access with clear delegation protocols (e.g., adult children managing Medicare, Medicaid, or Exchange accounts).
- Provides credentials for providers and navigators to view care plans and patient histories across networks.
- Supports privacy-preserving linkages of data across systems (e.g., through universal patient identifiers or identity resolution APIs).

Sharecare's AskMD has proposed a 'Verified Insurance ID' and credentialing mechanism that:

- Unifies a person's insurance eligibility, medical history, and coverage details into one real-time, interoperable profile, accessible anytime, anywhere
- Enables trusted access for caregivers through consented proxy models.
- Facilitates clean handoffs between AI triage, navigator support, and provider records, eliminating redundant intake and enabling seamless navigation.
- Improves attribution accuracy by tying member identity directly to validated plan enrollment and historical care patterns.

This identity layer enables:

- Patient matching across EHRs, payors, and third-party tools.
- Real-time care coordination even when beneficiaries move across providers.
- Audit trails for compliance and fraud prevention.
- More accurate cost accounting and risk adjustment by ensuring that quality metrics and diagnoses follow the correct patient identity across systems.

VB-15
How could implementing digital identity credentials improve value-based care delivery and outcomes?

Despite growing adoption of FHIR APIs and TEFCA-based data exchange, APM participants still face delays and inconsistencies in retrieving clinical and claims data across disparate health systems. The lack of a centralized, up-to-date directory of provider endpoints creates friction in record retrieval, impedes real-time care coordination, and limits the ability of digital tools to deliver context-aware, accurate recommendations at scale.

A nationwide FHIR endpoint directory should:

- Enable real-time discovery of where a provider's clinical or claims data can be accessed, including what resources (e.g., labs, care plans, medications) are supported.
- Support authentication workflows so tools can securely request and retrieve data via APIs without manual intervention.

- Map provider identifiers (NPIs) to their organizational affiliations, specialties, and data-sharing capabilities.
- Indicate whether the provider participates in TEFCA, or other national networks, enabling scalable, trust-based exchange.

Sharecare's AskMD and its HDS division rely on accurate endpoint data to automate release-of-information requests and retrieve complete longitudinal records across care settings. A nationwide directory would:

- Accelerate record retrieval workflows: Enabling AskMD to send API requests directly to the correct endpoint on first attempt, reducing administrative churn and patient delay.
- Improve claims data traceability: By linking NPIs in claims to known endpoint locations, digital tools can align utilization history with clinical context, improving attribution, analytics, and gap closure.
- Power smarter triage and risk assessment: AskMD's agentic AI platform could query specific endpoints (e.g., a specialist's medication list or a hospital's discharge summary) to tailor guidance or surface next best actions.

In addition, the directory should support federated identity and role-based access structures, enabling secure, permissioned retrieval based on user credentials (e.g. Login.gov, caregiver delegation) rather than static system permissions. This would allow digital health platforms like AskMD to access data on behalf of the right person, for the right purpose, using real-time identity verification protocols embedded in the directory framework.
Recommended directory fields:

- NPI and provider name
- FHIR endpoint URL(s)
- Supported FHIR resources (e.g., conditions, observations, CarePlan)
- TEFCA participation
- Authentication/authorization protocol (e.g., OAuth2)
- Organizational affiliation (e.g., system, group practice)
- Geographic service area
- Endpoint reliability/status metadata